EMS CLINICAL CHECKLISTS — BATCH 1

Paramedic level with EMT delegation · 1003 Non-Initiation of Care · 1004 Do Not Resuscitate · 2003 Asystole/PEA · 2005 V-Fib/Pulseless V-Tach · 2008 Suspected Cardiac Chest Pain

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

EDITION CHECK REQUIRED BEFORE ANY USE. The source file's cover says 'Last Update 07/01/2026'. Appendix A (PDF p.321) shows the 2026-2028 changes as Effective Nov. 1, 2026. 1001.1.h: after release of a new version, all previous versions are invalid. Confirm which edition is in force with agency clinical leadership (G-1).

SOURCE

Greater Miami Valley EMS Council standing orders, Regional Physician Advisory Board Region 3, file 2026protocol.pdf (328 pages). Page numbers = PDF viewer page index (1-328). Each protocol also prints its own 'Page x of y' footer; the printed footer is NOT used. Section numbers (e.g., 2008.2) are cited with every item.

CONTENTS OF THIS BATCH

Protocol	Title	Effective	Last modified	PDF pages used	Checkboxes	Not-specified lines
1003	Non-Initiation of Care	June 1, 2021	Feb. 11, 2024	PDF p.17	20	5
1004	Do Not Resuscitate	June 1, 2021	Jan. 19, 2025	PDF p.19	23	5
2003	Cardiac Arrest: Asystole or PEA	June 1, 2021	Oct. 10, 2021	PDF p.49 (+ 2001 p.45, 2002 p.47)	33	5
2005	Cardiac Arrest: V-Fib or Pulseless V-Tach	June 1, 2021	July 1, 2026	PDF pp.53-54 (+ 2001 p.45, 2002 p.47; monographs 8003, 8019, 8026, 8029)	44	4
2008	Suspected Cardiac Chest Pain	June 1, 2021	Feb. 13, 2023	PDF p.59 (+ 2009 p.61, 1014 p.41, 1005 pp.21-22; monographs 8004, 8021, 8034)	40	3

HOW TO READ THE TAGS

Tag	Meaning (source)
MCP ORDER	Source marks the item with a diamond. Per 1001.5.d.iv, diamond items ALWAYS require a physician's order (PDF p.12).
{ } OPTION	Source brackets the item. Per 1001.5.d.v, bracketed items are at the option of the agency and its Medical Director (PDF p.12).
EMR+ / EMT+ / AEMT+ / PM	Lowest level at which the source's colored level table lists the item. Levels are cumulative: the table lists directives 'for the EMT and above', etc. (1001.6, PDF p.13).
A / P / G	Source markers: A = adult only, P = pediatric (pink), G = geriatric (1001.5.d.vi-viii, PDF p.12).

WHAT THE DIAMOND AND BRACE MARKS DO NOT TELL YOU

Your department's Medical Director decides which { } options are adopted (1001.4.d: nothing in the protocol may be used without specific pre-approval of the local Medical Director). 1001.4.e-f: the protocol is used as clinically indicated; not every standing order applies to every patient, and discretionary judgment is required.

STRUCTURE OF THIS FILE

Cover; five checklists in the order above (each starts with its own title bar and status banner and follows the seven-heading template); REQUIRES HUMAN VERIFICATION (full text of every gap and conflict); SOURCE-VERIFICATION TABLES (one row per checklist line). Use the companion HTML file for collapsible, tappable checklists.

AUDIT SUMMARY

Automated source audit at build: 168 checklist items — every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures); 176 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 22 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

Contrast check of the print palette (WCAG ratio): navy on white 11.48:1; body on white 17.06:1; teal on white 4.99:1; red on white 5.44:1; cream on navy 10.42:1; white on navy 11.48:1.

GENERAL PROTOCOL 1003 — NON-INITIATION OF CARE

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Non-Initiation of Care	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER General Protocol 1003 — Non-Initiation of Care	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Feb. 11, 2024
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.17) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

WHO MAY APPLY / SCOPE (1003.1)

☐	**EMT+**	Protocol may be applied by EMT, AEMT and Paramedic only. The EMR cannot determine that a patient is deceased.	1003.1.a · p.17
☐	**ALL**	All patients (adult, pediatric, geriatric) may meet non-initiation criteria.	1003.1.b · p.17

RESUSCITATION WILL NOT BE INITIATED IF (1003.2.A)

☐	**EMT+**	Deep, penetrating, cranial injuries.	1003.2.a.i · p.17
☐	**EMT+**	Massive truncal wounds.	1003.2.a.ii · p.17
☐	**EMT+**	DNR order present and valid (see 1004 Do Not Resuscitate).	1003.2.a.iii · p.17
☐	**EMT+**	Frozen body.	1003.2.a.iv · p.17
☐	**EMT+**	Rigor mortis, tissue decomposition, or severe dependent lividity.	1003.2.a.v · p.17
☐	**EMT+**	Triage demands.	1003.2.a.vi · p.17
☐	**EMT+**	Arrest from BLUNT OR PENETRATING TRAUMA: CONSIDER not initiating care for injuries obviously incompatible with life. Considerations listed: prolonged arrest (greater than 10 minutes); possibility of MIXED MECHANISMS. *Condition / note: 'Consider' language: discretionary, not mandatory.*	1003.2.a.vii · p.17

EXCLUSIONARY CONDITIONS: THESE WILL NOT MEET NON-INITIATION CRITERIA (1003.3.A)

☐	**EMT+**	Traumatic arrest in a female patient with known pregnancy greater than 24 weeks OR uterine fundus palpable at or above the umbilicus.	1003.3.a.i · p.17
☐	**EMT+**	Possible medical etiology for the traumatic cardiac arrest.	1003.3.a.ii · p.17
☐	**EMT+**	Arrest witnessed by EMS providers.	1003.3.a.iii · p.17
☐	**EMT+**	Lightning strike.	1003.3.a.iv · p.17
☐	**EMT+**	Signs or symptoms of a hypothermic patient.	1003.3.a.v · p.17
☐	**EMT+**	Focused blunt trauma to the chest (commotio cordis).	1003.3.a.vi · p.17

2 ASSESSMENT

☐	**EMT+**	Decision threshold in trauma arrest: 'prolonged arrest' is defined as greater than 10 minutes. *Condition / note: Starting point of the 10 minutes is not defined (see U-1003-2).*	1003.2.a.vii.1 · p.17

Required primary / focused assessment, history, exam findings or measurements: NOT SPECIFIED IN SOURCE — VERIFY. 1003 lists criteria only; it gives no assessment procedure.

3 TREATMENT / CRITICAL ACTIONS

☐	**EMT+**	If a 1003.2 criterion is met and no 1003.3 exclusionary condition applies: do not initiate resuscitation. *Condition / note: Interaction of 1003.2 and 1003.3 is not spelled out (see U-1003-1).*	1003.2.a · p.17 1003.3.a · p.17
☐	**EMT+**	If care has already begun and it is readily apparent the patient meets non-initiation criteria: resuscitation efforts MAY cease.	1003.1.c · p.17

Medication / procedure details: NOT SPECIFIED IN SOURCE — VERIFY (1003 specifies none).

EMT DELEGATION — from the source level tables	
MAY	EMT may apply the protocol and determine that criteria are met (1003.1.a).
NOTE	The EMR may NOT determine that a patient is deceased (1003.1.a).

4 REASSESSMENT / RESPONSE

Reassessment, response criteria, worsening pathway: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	ALL	Organ donation inquiry: direct the call to Life Connection of Ohio at 1-800-535-9206.	1003.4 · p.17
☐	ALL	Reasons to contact Medical Control listed in 1002.2.b include 'field termination or DNR clarification'. *Condition / note: 1003 itself does not require MCP contact for non-initiation (see U-1003-3).*	1002.2.b · p.15

Transport / destination, receiving-facility notification and handoff content for non-initiation: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout elements for non-initiation: NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-1003-1	1003.3.a / 1003.2.a (p.17)	Exclusionary conditions 'will not meet non-initiation criteria' but the text does not say whether they override...
U-1003-2	1003.2.a.vii.1 (p.17)	'Prolonged arrest (greater than 10 minutes)' has no stated starting point (collapse, 911 call, EMS arrival?).
U-1003-3	1003.1.c / 1002.2.b (p.17)	1003 allows resuscitation to cease without stating MCP contact; 2001 requires MCP approval for field termination
U-1003-4	1003.2.a.vi (p.17)	'Triage demands' is not defined in 1003 (SALT triage 3019 and Crisis Standard of Care 3021 exist; not reviewed for...
U-1003-5	1003 (whole) (p.17)	1003 contains no documentation, assessment, notification or scene-disposition instructions

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

GENERAL PROTOCOL 1004 — DO NOT RESUSCITATE

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Do Not Resuscitate	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER General Protocol 1004 — Do Not Resuscitate	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Jan. 19, 2025
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.19) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

VALID ORDERS (1004.1, 1004.2)

☐	ALL	Consider and honor all valid Ohio Do Not Resuscitate orders (ORC 2133.01-2133.26).	1004.1.a · p.19
☐	ALL	The two valid DNR orders are DNR: Comfort Care and DNR: Comfort Care Arrest.	1004.1.b · p.19
☐	ALL	DNR Comfort Care Arrest (DNR-CCA): permits any GMVEMSC Protocol treatment until the patient goes into cardiac or respiratory arrest. Once in arrest, only the permitted DNR treatment is performed.	1004.2.a · p.19
☐	ALL	DNR Comfort Care (DNR-CC): permits any medical treatment to diminish pain or discomfort; no treatment should be used to postpone the patient's death; the order is initiated at the moment it is signed by the patient's physician.	1004.2.b · p.19

PRECEDENCE AND SPECIAL SITUATIONS (1004.4)

☐	ALL	If more than one living will declaration or DNR exists, the most recent supersedes the previous.	1004.4.a · p.19
☐	ALL	The authority of a DPOA-HC supersedes the DNR if the DPOA-HC previously consented to the DNR.	1004.4.b · p.19
☐	ALL	Out-of-state DNR orders and pediatric DNR orders are recognized as valid special situations: contact MCP and request to honor the DNR with physician permission.	1004.4.c · p.19

2 ASSESSMENT

☐	ALL	Conduct an initial assessment (permitted once DNR is initiated).	1004.3.a.i · p.19

How a DNR is recognized as 'present and valid' (document type, form features, identification, who may present it): NOT SPECIFIED IN SOURCE — VERIFY.

Required history, exam findings, measurements and thresholds: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

PERMITTED ONCE A DNR IS INITIATED (1004.3.A)

☐	**ALL**	Perform basic medical care.	1004.3.a.ii · p.19
☐	**ALL**	Clear airway of obstruction or suction.	1004.3.a.iii · p.19
☐	**ALL**	If necessary, for comfort or to relieve distress: oxygen, CPAP or BiPAP. *Condition / note: BiPAP appears as an agency-option item elsewhere (Appendix A, 1007.1) — see U-1004-3.*	1004.3.a.iv · p.19
☐	**ALL**	If necessary, IV access for hydration or pain medication to relieve discomfort, NOT to postpone death.	1004.3.a.v · p.19
☐	**ALL**	If possible, contact other appropriate health care providers.	1004.3.a.vi · p.19
☐	**ALL**	Blood glucose checks and treatment of 4008 Diabetic Emergencies - Hypoglycemia/Hyperglycemia are acceptable even with a valid DNR.	1004.4.d · p.19

NOT PERMITTED ONCE AN ORDER IS VALID AND EFFECTIVE (1004.3.B)

☐	**ALL**	Do NOT perform CPR.	1004.3.b.i · p.19
☐	**ALL**	Do NOT administer resuscitation medications with the intent of restarting the heart or breathing.	1004.3.b.ii · p.19
☐	**ALL**	Do NOT insert an airway adjunct.	1004.3.b.iii · p.19
☐	**ALL**	Do NOT defibrillate, cardiovert or initiate pacing.	1004.3.b.iv · p.19
☐	**ALL**	Do NOT initiate continuous cardiac monitoring.	1004.3.b.v · p.19
☐	**ALL**	1005 requires continuous cardiac monitoring when administering pain medications; 1004 supersedes that requirement in valid DNR patients.	1004.4.e · p.19

EMT DELEGATION — from the source level tables

NOTE	1004 states no provider-level differences. 1003.1.a limits determination that a patient is deceased to EMT, AEMT and Paramedic (EMR may not).

4 REASSESSMENT / RESPONSE

Reassessment timing, response criteria and worsening pathway: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

☐	**ALL**	Questions about the documents: try to keep the patient's intent in mind.	1004.4.f · p.19
☐	**ALL** **MCP ORDER**	Any confusion on scene: call MCP for clarification.	1004.4.g · p.19
☐	**ALL**	Reasons to contact Medical Control include 'field termination or DNR clarification'.	1002.2.b · p.15

Transport / destination and handoff content for DNR patients: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Documentation / closeout (copy of order, times, who was contacted): NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

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Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-1004-1	1004.1 / 1003.2.a.iii (p.19)	1003 uses 'DNR order present and valid' but 1004 does not describe what makes an order present or valid.
U-1004-2	1004.2.a vs .b (p.19)	DNR-CC states it is initiated 'at the moment it is signed by the patient's physician'; no equivalent start statement...
U-1004-3	1004.3.a.iv (p.19)	BiPAP is listed as permitted for comfort; Appendix A (PDF p.321, 1007.1) shows BiPAP as a bracketed, agency-option...
U-1004-4	1004.4.b (p.19)	'DPOA-HC supersedes the DNR if the DPOA-HC previously consented to the DNR' is ambiguous about what happens when the...
U-1004-5	1004.4.c (p.19)	Out-of-state and pediatric DNR orders: pediatric is defined as less than 16 years (1005.4); 1004 does not state how...
U-1004-6	1004 (whole) (p.19)	1004.4.g marks 'Call MCP for clarification' with a diamond (MCP order/contact)

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

CARDIAC PROTOCOL 2003 — CARDIAC ARREST: ASYSTOLE OR PEA

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Cardiac Arrest: Asystole or PEA	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Cardiac Protocol 2003 — Cardiac Arrest: Asystole or PEA	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Oct. 10, 2021
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.49 (+ 2001 p.45, 2002 p.47)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION (2003.2 ASSESSMENT)

☐	**EMR+**	Unresponsive; pulseless and apneic.	2003.2 Signs & Symptoms · p.49
☐	**EMR+**	EITHER no electrical activity on the cardiac monitor OR electrical activity on the monitor with no pulse present.	2003.2 Signs & Symptoms · p.49
	ALL	Listed differential: ventricular fibrillation; pulseless ventricular tachycardia; other causes of unresponsiveness; device (lead) error; signs of irreversible death.	2003.2 Differential · p.49

IMMEDIATE PRIORITIES / ESCALATION

☐	**EMR+**	In ALL cardiac arrest patients apply 2002 Cardiac Arrest: Basic Life Support. Apply the appropriate guideline after rhythm interpretation.	2003.1.a-b · p.49
☐	**EMR+**	Rhythms may change: be flexible to move between the different protocols.	2003.1.c · p.49
☐	**ALL**	Resuscitation applies when the patient does NOT meet the Non-initiation of Care guideline (see 1003).	2001.2 Assessment · p.45
☐	**ALL**	Pediatric: dosing should never exceed adult doses. Pediatric = patients less than 16 years old.	2003.2 Ped. Considerations · p.49 1005.4 Ped. Considerations · p.21

2 ASSESSMENT

☐	**EMR+**	Apply the AED and check for a shockable rhythm.	2003.2 Tx Algorithm, EMR block · p.49
☐	**ALL**	Apply 1005 General Patient Management to all patients (not reproduced here). *Condition / note: 1005 content is NOT reproduced in this checklist (see G-3).*	1005.1.a · p.21

Required history, exam findings, measurements and numeric thresholds beyond the rhythm check: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

BLS FOUNDATION — ALL PROVIDERS (2002 BLS APPLIES; ADULT VALUES SHOWN)

☐	**EMR+**	If witnessed or unwitnessed arrest: initiate quality CPR for up to 2 minutes. *Condition / note: 2002 and 2005 state '1-2 minutes' (see U-2003-1).*	2003.2 Tx Algorithm, EMR block · p.49
☐	**EMR+**	Follow the 2002 Cardiac Arrest - BLS protocol.	2003.2 Tx Algorithm, EMR block · p.49
☐	**EMR+**	If no defibrillation is indicated: continuous CPR.	2003.2 Tx Algorithm, EMR block · p.49
☐	**EMR+**	CPR parameters (adult): compressions 100 to 120 per minute; depth at least 2 inches; 30:2 without an advanced airway; with an advanced airway continuous compressions with 1 breath every 6 seconds; limit interruptions to less than 10 seconds. *Condition / note: Pediatric, infant and newborn values: see 2002.1 table (not reproduced).*	2002.1 table · p.47

EMT LEVEL AND ABOVE

☐	**EMT+** **{ } OPTION**	Obtain and transmit {12 Lead EKG} if the patient has ROSC.	2003.2 Tx Algorithm, EMT block · p.49

AEMT LEVEL AND ABOVE

☐	**AEMT+**	Consider possible causes.	2003.2 Tx Algorithm, AEMT block · p.49
☐	**EMR+**	Consider ACLS treatable causes (Hs & Ts) to your level of certification. As listed — EMR: hypoxia, hypothermia. EMT: toxins. AEMT: hypovolemia, hydrogen ion, tension pneumothorax. Paramedic: tamponade (cardiac), thrombosis (coronary, pulmonary). *Condition / note: Cumulative application for the Paramedic is implied, not stated (see U-2003-5).*	2002.2 Clinical Pearls · p.47

PARAMEDIC

☐	PM	ADULT: Epinephrine (1:10,000) 1 mg IV or IO, repeat every 3-5 minutes. *Condition / note: 8019 dosing line lists 'IV' only (see U-2003-4).*	2003.2 Tx Algorithm, Paramedic block · p.49 8019 Adult Dosing · p.261
☐	PM	PEDIATRIC: Epinephrine (1:10,000) 0.01 mg/kg IV or IO, repeat every 3-5 minutes (8019: max single dose 1 mg).	2003.2 Tx Algorithm, Paramedic block · p.49 8019 Pediatric Dosing · p.261
	PM	8019 Epinephrine 1:10,000: indicated 'For the Paramedic only' for VF, pulseless VT, asystole and PEA in all patients. Contraindications: none in the emergency setting. Medical control: adults No; pediatrics No.	8019 Indications / Contraindications / Medical Control · p.261
☐	PM	The Paramedic may consider Field Termination after administering Epinephrine. *Condition / note: Field termination requires MCP approval and criteria in 2001 (see Section 5).*	2003.2 Tx Algorithm, Paramedic block · p.49
☐	PM	Paramedics are expected to provide ALS resuscitative care at the scene; prioritize on-scene resuscitation efforts, regardless of age, unless clinically indicated.	2001.1.b-c · p.45

EMT DELEGATION — from the source level tables

MAY	EMT: continues resuscitation until patient is handed off to a higher-level provider; if no higher-level provider is available, then transport (2001.2 EMT block, PDF p.45).
MAY	EMT: AED / BLS per 2002; obtain and transmit {12 Lead EKG} if ROSC {agency option} (2002.2 EMT block, 2003.2).
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: endotracheal intubation; IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG} for cardiac monitoring; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
MCP ORDER	If no ALS equipment is available at the scene and transport time to a medical facility will exceed 20 minutes, field termination may be considered (2001.2 EMT block, PDF p.45).
NOT LISTED AT EMT LEVEL	Epinephrine for cardiac arrest (8019 'For the Paramedic only'); consideration of field termination (listed from AEMT level in 2001/2003 except the EMT diamond item above).

4 REASSESSMENT / RESPONSE

☐	EMR+	Change the person compressing the chest every 2 minutes.	2002.2 Clinical Pearls · p.47
☐	EMR+	If ROSC: follow 2001 Resuscitation Guidelines.	2003.1.d · p.49
☐	AEMT+	Prolonged resuscitation efforts are required if: PEA with a rate greater than 40 per minute; OR upward trending or persistent EtCO2 greater than or equal to 20 mmHg, refractory to VF or VT. *Condition / note: 'Refractory to VF or VT' wording (see U-2003-3).*	2001.2 Tx Algorithm, AEMT block · p.45

Required reassessment timing (pulse/rhythm-check interval) other than 'CPR for up to 2 minutes' and the epinephrine repeat interval: NOT SPECIFIED IN SOURCE — VERIFY.

Time to first epinephrine dose and criteria for stopping epinephrine: NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

TRANSPORT / DESTINATION

☐	PM	Cardiac arrests should NOT be transported unless: when indicated, at least one defibrillation has been delivered; ROSC; the airway cannot be secured; vascular access is not established; or MCP declines to authorize Field Termination.	2002.2 Tx Algorithm, Paramedic block · p.47
☐	EMT+	ROSC: transport to an interventional facility if transport time is less than 30 minutes.	2001.2 EMT block · p.45 2002.2 EMT block · p.47
☐	PM	A documented STEMI AND a witnessed cardiac arrest: transport to a cardiac interventional facility if transport time is less than 30 minutes.	2001.2 Paramedic block · p.45
☐	AEMT+	Arrest due to profound hypothermia: rapidly transport to a Trauma Center.	2001.2 AEMT block · p.45
☐	ALL	Consider aeromedical transport for transports greater than 30 minutes if the patient has ROSC.	2001.2 Clinical Pearls · p.45

FIELD TERMINATION — ADULT ONLY

☐	ALL	FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS.	2001.2 Ped. Considerations · p.45
☐	AEMT+ MCP ORDER	Field termination requires MCP approval and may only be considered when the following criteria are met: 18 years or older; asystole or PEA with rates less than 40; not in arrest due to hypothermia; advanced airway in place; vascular access in place; no signs of neurological function such as reactive pupils, response to pain or spontaneous movement.	2001.2 AEMT block · p.45
☐	AEMT+	When the AEMT or Paramedic contacts MCP for consent for field termination, provide: duration of the resuscitation; how long the patient may have been in arrest prior to EMS arrival; witnessed or unwitnessed; current EtCO2; presenting rhythm.	2001.2 Consult · p.45

NOTIFICATION / CONSULTATION

☐	**AEMT+**	No consult required unless applying the Field Termination Guideline. The AEMT or Paramedic may consult MCP to field terminate.	2003.2 Consult · p.49
☐	**ALL**	Contact ED to request a Cardiac Alert if applicable. When calling with an Alert: request to speak directly to the MCP after a brief summary; state 'We recommend a ____ Alert'; the MCP may withhold the Alert.	2003.2 Consult · p.49 1002.3.c.ii · p.15
☐	**ALL**	Contact the receiving hospital prior to arrival with a cardiac arrest patient.	2003.2 Clinical Pearls · p.49 1002.1.a.i · p.15

Required handoff / transfer-of-care content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Required documentation elements, times, refusals, signatures: NOT SPECIFIED IN SOURCE — VERIFY (2003 and 2002 contain no documentation directives; 2001 lists only the information to give MCP, Section 5).

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-2003-1	2003.2 vs 2002.2 / 2005.2 (p.49)	2003 says CPR 'for up to 2 minutes'; 2002 and 2005 say '1-2 minutes'
U-2003-2	2003.2 Paramedic block vs 2001.2 (p.49)	2003: 'Paramedic may consider Field Termination after administering Epinephrine' and 'no consult required unless...
U-2003-3	2001.2 AEMT block (p.45)	'Upward trending or persistent EtCO2 >= 20 mmHg, refractory to VF or VT' is written for VF/VT, yet is listed in a...
U-2003-4	2003.2 vs 8019 (p.261)	2003 says epinephrine 'IV or IO'; the 8019 dosing lines list 'IV' only
U-2003-5	2002.2 Pearls (p.47)	Hs & Ts are listed per certification level; whether the Paramedic applies all rows (cumulative) is not stated...
U-2003-6	2001.2 / 2003.2 (p.45)	'Pediatric' is <16 years (1005.4); field termination requires 18 years or older (2001)
U-2003-7	2002.2 / 1008 / 1012 (p.47)	2002 conditions transport on the airway being secured and vascular access established; airway (1008/1009) and IO...

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

CARDIAC PROTOCOL 2005 — CARDIAC ARREST: V-FIB OR PULSELESS V-TACH

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Cardiac Arrest: V-Fib or Pulseless V-Tach	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Cardiac Protocol 2005 — Cardiac Arrest: V-Fib or Pulseless V-Tach	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified July 1, 2026
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF pp.53-54 (+ 2001 p.45, 2002 p.47; monographs 8003, 8019, 8026, 8029)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION (2005.2 ASSESSMENT)

☐	**EMR+**	Unresponsive; pulseless and apneic; ventricular fibrillation or ventricular tachycardia on cardiac monitor or AED.	2005.2 Signs & Symptoms · p.53
	ALL	Listed differential: asystole; artifact/device failure; signs of irreversible death; other causes of unresponsiveness.	2005.2 Differential · p.53

IMMEDIATE PRIORITIES / ESCALATION

☐	**EMR+**	In ALL cardiac arrest patients apply 2002 Cardiac Arrest: Basic Life Support. Apply the appropriate guideline after rhythm interpretation. Rhythms may change: move between the different protocols.	2005.1.a-c · p.53
☐	**EMR+**	If ROSC: follow 2001 Resuscitation Guidelines.	2005.1.d · p.53
☐	**ALL**	Resuscitation applies when the patient does NOT meet the Non-initiation of Care guideline (see 1003).	2001.2 Assessment · p.45
☐	**ALL**	Pediatric: dosing should never exceed adult doses. Pediatric = patients less than 16 years old.	2005.2 Ped. Considerations · p.53 1005.4 Ped. Considerations · p.21

SOURCE DEFINITIONS THAT DRIVE DECISIONS (2005.3)

☐	**AEMT+**	REFRACTORY V-Fib/PVT = NOT CONVERTED by three standard defibrillations.	2005.3.d.ii · p.54
☐	**AEMT+**	RECURRENT V-Fib/PVT = V-Fib/PVT that reoccurs episodically after successful conversion with intervening episodes of organized electrical activity. Vector Change and Double Sequential Defibrillation are NOT indicated.	2005.3.d.vi · p.54 2005.3.f · p.54

2 ASSESSMENT

☐	**EMR+**	Apply the AED / monitor and defibrillate as indicated by the rhythm.	2005.2 Tx Algorithm, EMR block · p.53
☐	**AEMT+**	AEMT level and above: defibrillate as required based on EKG interpretation.	2005.2 Tx Algorithm, AEMT block · p.53
☐	**PM**	Post-ROSC amiodarone decision point: do not infuse unless SBP is greater than 100.	2005.2 Tx Algorithm, Paramedic block · p.53
☐	**ALL**	Apply 1005 General Patient Management to all patients (not reproduced here). *Condition / note: 1005 content is NOT reproduced (see G-3).*	1005.1.a · p.21

Required history, exam findings and measurements beyond rhythm identification: NOT SPECIFIED IN SOURCE — VERIFY.

3 TREATMENT / CRITICAL ACTIONS

ALL PROVIDERS (EMR BLOCK, 2005.2)

☐	**EMR+**	If witnessed or unwitnessed arrest: initiate quality CPR for 1-2 minutes and proceed to first defibrillation.	2005.2 Tx Algorithm, EMR block · p.53
☐	**EMR+**	Follow the Basic Life Support protocol (2002).	2005.2 Tx Algorithm, EMR block · p.53
☐	**EMR+**	Energy settings (adult): for initial and subsequent defibrillations, follow the manufacturer recommendation.	2005.2 Clinical Pearls · p.54
☐	**EMR+**	PEDIATRIC energy: start at 2 J/kg (or biphasic equivalent) and increase by 2 J/kg (or biphasic equivalent) each shock; maximum pediatric shock 10 J/kg (or biphasic equivalent).	2005.2 Clinical Pearls · p.54

EMT LEVEL AND ABOVE

☐	**EMT+** **{ } OPTION**	Obtain and transmit {12 Lead EKG} if the patient has ROSC.	2005.2 Tx Algorithm, EMT block · p.53

AEMT LEVEL AND ABOVE

☐	**AEMT+**	Consider possible causes (Hs & Ts per 2002 Pearls; see 2003 checklist item).	2005.2 Tx Algorithm, AEMT block · p.53 2002.2 Clinical Pearls · p.47
☐	**AEMT+** **{ } OPTION**	ADULT: for refractory V-Fib/PVT after three shocks, consider Vector Change Defibrillation for subsequent shocks. *Condition / note: Optional skill; NOT to be applied without explicit Medical Director consent; adults only (2005.3.a-c).*	2005.2 Tx Algorithm, AEMT block · p.53 2005.3.d · p.54

PARAMEDIC — SEQUENCE (2005.2 PARAMEDIC BLOCK)

☐	**PM**	Alternate between CPR / defibrillation / medication administration.	2005.2 Tx Algorithm, Paramedic block · p.53
☐	**PM**	ADULT: Epinephrine 1 mg 1:10,000 IV or IO, repeat every 3-5 minutes. *Condition / note: 8019 dosing line lists 'IV' only (see U-2005-8).*	2005.2 Tx Algorithm, Paramedic block · p.53 8019 Adult Dosing · p.261
☐	**PM**	PEDIATRIC: Epinephrine (1:10,000) 0.01 mg/kg IV or IO, repeat every 3-5 minutes (8019: max single dose 1 mg).	2005.2 Tx Algorithm, Paramedic block · p.53 8019 Pediatric Dosing · p.261
☐	**PM**	AFTER THIRD DEFIBRILLATION — ADULT: Amiodarone 300 mg IV or IO.	2005.2 Tx Algorithm, Paramedic block · p.53 8003 Adult Dosing · p.229
☐	**PM**	AFTER THIRD DEFIBRILLATION — PEDIATRIC: Amiodarone 5 mg/kg IV or IO (max first dose 300 mg).	2005.2 Tx Algorithm, Paramedic block · p.53 8003 Pediatric Dosing · p.229
☐	**PM**	If Amiodarone is NOT available after third defibrillation — ADULT: Lidocaine 150 mg IV or IO; PEDIATRIC: Lidocaine 1.0 mg/kg IV or IO (max first dose 100 mg). *Condition / note: 'Not available' is not defined (see U-2005-2).*	2005.2 Tx Algorithm, Paramedic block · p.53 8026 Adult/Pediatric Dosing · p.275
☐	**PM**	ADULT, evidence of pulseless polymorphic VT (torsades de pointes): Magnesium Sulfate 2 gm infused with macro-drip tubing over 10 minutes.	2005.2 Tx Algorithm, Paramedic block · p.53 8029 Adult Dosing · p.281
☐	**PM** **{ } OPTION**	ADULT: after three traditional defibrillations and at least one antiarrhythmic medication, consider Double Sequential Defibrillation for subsequent shocks. *Condition / note: Optional skill; NOT to be applied without explicit Medical Director consent; adults only (2005.3.a-c).*	2005.2 Tx Algorithm, Paramedic block · p.53 2005.3.e · p.54

☐	**PM**	AFTER SIXTH DEFIBRILLATION — ADULT: Amiodarone 150 mg IV or IO. PEDIATRIC: Amiodarone 5 mg/kg IV or IO (max first dose 150 mg). *Condition / note: 8003: repeat 'no sooner than 10 minutes after first dose' (see U-2005-1).*	2005.2 Tx Algorithm, Paramedic block · p.53 8003 Dosing · p.229
☐	**PM**	If Amiodarone is NOT available after sixth defibrillation — ADULT: Lidocaine 75 mg IV or IO; PEDIATRIC: Lidocaine 1.0 mg/kg IV or IO (max first dose 75 mg).	2005.2 Tx Algorithm, Paramedic block · p.53 8026 Dosing · p.275

ALTERNATE DEFIBRILLATION TECHNIQUES (2005.3) — ADULT ONLY, OPTIONAL

☐	**AEMT+**	Vector Change and Double Sequential Defibrillation are optional skills in the GMVEMSC Protocol. Providers SHOULD NOT apply them without the explicit consent of their Medical Director. Approved for adult patients only.	2005.3.a-c · p.54
☐	**AEMT+**	VECTOR CHANGE (AEMT and Paramedic): for refractory V-Fib/PVT only. Place a second set of defib pads in an anterior-posterior position with minimal interruption in CPR; subsequent defibrillations through the anterior-posterior pads. NOT for recurrent V-Fib/PVT.	2005.3.d.i-vi · p.54
☐	**PM**	DOUBLE SEQUENTIAL DEFIBRILLATION (Paramedic): for refractory V-Fib/PVT following three standard defibrillations and at least one round of an antiarrhythmic (amiodarone or lidocaine). Requires two manual biphasic defibrillators; one set of pads anterior-apical (traditional) and one anterior-posterior; both machines charged to maximum energy with discharge as simultaneous as possible; repeat as indicated; all subsequent defibrillations double sequential.	2005.3.e.i-v · p.54
☐	**PM**	DSD CAUTION: every agency considering this procedure needs to consult the manufacturer of its cardiac monitor; the technique is 'off-label'; Agency Administrators should be made aware of the risk to monitors.	2005.3.e.vi-vii · p.54

DRUG MONOGRAPH DETAILS (VERIFIED AGAINST 8003, 8019, 8026, 8029)

PM	8003 AMIODARONE: adult 300 mg IV/IO, may repeat 150 mg IV/IO no sooner than 10 minutes after first dose; pediatric 5 mg/kg IV/IO (max first dose 300 mg), may repeat 5 mg/kg no sooner than 10 minutes, max repeat dose 150 mg. Listed contraindications: pulmonary congestion; cardiogenic shock; hypotension (SBP less than 100); sensitivity to amiodarone. Continuous EKG monitoring is required. Medical control: adult No; pediatric No.	8003 · p.229
PM	8026 LIDOCAINE (cardiac arrest V-Fib/Pulseless V-Tach, in the absence of Amiodarone; source labels this JITSO): adult 150 mg (7.5 ml) IV/IO, repeat 75 mg (3.75 ml) IV/IO; pediatric 1 mg/kg IV/IO (max 100 mg), repeat 1 mg/kg (max 75 mg). Listed contraindications: hypersensitivity; second or third degree heart block in absence of an artificial pacemaker. Medical control: adult No; pediatric No.	8026 · p.275
PM	8029 MAGNESIUM SULFATE (polymorphic VT / torsades, adult, with or without a pulse): 2 gm (1/2 IV bag) with macro-drip tubing over 10 min; 4 gm in 100 ml bag (40 mg/ml). Listed contraindications: hypersensitivity; any heart block except 1st degree; administer with caution if SBP less than 90 mmHg; acute myocardial infarction (AMI); abdominal pain. Continuous cardiac monitoring required. Medical control: adult Yes only when given for refractory asthma.	8029 · p.281

EMT DELEGATION — from the source level tables

MAY	EMT (EMR block): quality CPR for 1-2 minutes; follow BLS; defibrillate as indicated by the AED (2005.2, PDF p.53).
MAY	EMT: obtain and transmit {12 Lead EKG} if ROSC {agency option} (2005.2 EMT block, PDF p.53).
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: endotracheal intubation; IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21).
MCP ORDER	If no ALS equipment is available at the scene and transport time to a medical facility will exceed 20 minutes, field termination may be considered (2001.2 EMT block, PDF p.45).
NOT LISTED AT EMT LEVEL	Manual defibrillation by EKG interpretation, Vector Change, Double Sequential Defibrillation (AEMT / Paramedic blocks); epinephrine, amiodarone, lidocaine, magnesium sulfate (Paramedic block; 8019 'For the Paramedic only').

4 REASSESSMENT / RESPONSE

☐	**EMR+**	Resume chest compressions immediately following each defibrillation, without performing pulse check, for 1-2 minutes.	2005.2 Clinical Pearls · p.54
☐	**EMR+**	Minimize interruptions to compressions before and after each shock to less than 10 seconds; change the person compressing every 2 minutes.	2002.2 Clinical Pearls · p.47
☐	**PM**	If patient converts with ROSC from a ventricular arrhythmia and NO antiarrhythmic has been given — ADULT: Amiodarone 150 mg in 250 ml NS IV over 10 minutes using 60 drop/ml tubing. Do NOT infuse unless SBP is greater than 100. Consider IV fluid 500 ml to increase SBP to 100 or higher prior to infusion. *Condition / note: 8003 adds 'wide open' and '18 g angiocath'; 2005 does not (see U-2005-4).*	2005.2 Tx Algorithm, Paramedic block · p.53 8003 Adult Dosing · p.229
☐	**EMT+ { } OPTION**	If ROSC: follow 2001 Resuscitation Guidelines; obtain and transmit {12 Lead EKG} (EMT level, agency option).	2005.1.d, 2005.2 EMT block · p.53

☐	**AEMT+**	Refractory V-Fib/PVT (three standard defibrillations without conversion): see Section 3 for Vector Change (AEMT/PM) and Double Sequential Defibrillation (PM), both optional and adult-only.	2005.3.d-e · p.54

Required reassessment timing other than the intervals above; criteria to stop resuscitation other than field termination (Section 5): NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

TRANSPORT / DESTINATION

☐	**PM**	Cardiac arrests should NOT be transported unless: when indicated, at least one defibrillation has been delivered; ROSC; the airway cannot be secured; vascular access is not established; or MCP declines to authorize Field Termination.	2002.2 Tx Algorithm, Paramedic block · p.47
☐	**EMT+**	ROSC: transport to an interventional facility if transport time is less than 30 minutes. A documented STEMI AND a witnessed cardiac arrest: same.	2001.2 EMT and Paramedic blocks · p.45 2002.2 EMT block · p.47
☐	**ALL**	Consider aeromedical transport for transports greater than 30 minutes if the patient has ROSC.	2001.2 Clinical Pearls · p.45
☐	**AEMT+ MCP ORDER**	FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS. For adults, field termination requires MCP approval and the 2001.2 criteria (see 2003 checklist, Section 5). Criteria name asystole or PEA with rates less than 40. *Condition / note: 2005 Consult mentions field termination but 2001 criteria name asystole/PEA only (see U-2005-5).*	2001.2 Ped. Considerations / AEMT block · p.45

NOTIFICATION / CONSULTATION

☐	**AEMT+**	No consult required unless applying the Field Termination Guideline. The AEMT or Paramedic may consult MCP to field terminate.	2005.2 Consult · p.54
☐	**ALL**	Contact ED to request a Cardiac Alert if applicable. When calling with an Alert: request to speak directly to the MCP after a brief summary; state 'We recommend a ____ Alert'; the MCP may withhold the Alert.	2005.2 Consult · p.54 1002.3.c.ii · p.15
☐	**ALL**	Contact the receiving hospital prior to arrival with a cardiac arrest patient.	2005.2 Clinical Pearls · p.54 1002.1.a.i · p.15

Required handoff / transfer-of-care content: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

Required documentation elements, shock counts, times, refusals, signatures: NOT SPECIFIED IN SOURCE — VERIFY (2005 contains no documentation directives). Medical Director consent for Vector Change / DSD (2005.3.b) is a record the agency may need to confirm.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-2005-1	2005.2 vs 8003 (p.229)	2005 triggers amiodarone 300 mg after the THIRD defibrillation and 150 mg after the SIXTH; the 8003 monograph says...
U-2005-2	2005.2 Paramedic block (p.53)	'If Amiodarone is not available, use Lidocaine' — 'not available' is undefined
U-2005-3	8003, 8029 Contraindications (p.229)	8003 lists contraindications (pulmonary congestion, cardiogenic shock, hypotension SBP less than 100)
U-2005-4	2005.2 vs 8003 (p.229)	Post-ROSC amiodarone: 2005 says 'IV over 10 minutes using 60 drop/ml tubing'; 8003 adds 'wide open' and '18 g...
U-2005-5	2005.2 Consult vs 2001.2 (p.54)	2005 Consult says the AEMT/Paramedic may consult MCP to field terminate, but 2001 criteria permit field termination...
U-2005-6	2005.3.a-c (p.54)	Vector Change and Double Sequential Defibrillation are bracketed {optional}; use requires explicit Medical Director...
U-2005-7	2005.2 (p.53)	Not specified: adult energy values (source says follow manufacturer); whether DSD/Vector Change shocks count toward...
U-2005-8	2005.2 vs 8019 (p.261)	2005 says epinephrine 'IV or IO'; 8019 dosing lines list 'IV' only (1005.4 Pearls: any IV medication can be given IO).
U-2005-9	2002.2 / 1008 / 1012 (p.47)	Airway (1008/1009) and IO (1012) procedures, and 2010/2011 rhythm protocols, are NOT reviewed or reproduced.

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

CARDIAC PROTOCOL 2008 — SUSPECTED CARDIAC CHEST PAIN

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

CHECKLIST / CONDITION Suspected Cardiac Chest Pain	**PROVIDER LEVEL** [X] Paramedic [ ] EMT [ ] AEMT [ ] All — EMT delegation in Section 3
PROTOCOL TITLE / NUMBER Cardiac Protocol 2008 — Suspected Cardiac Chest Pain	**PROTOCOL REVISION / EFFECTIVE DATE** Effective June 1, 2021 · Last modified Feb. 13, 2023
SOURCE FILE / ISSUING AUTHORITY 2026protocol.pdf (PDF p.59 (+ 2009 p.61, 1014 p.41, 1005 pp.21-22; monographs 8004, 8021, 8034)) · Greater Miami Valley EMS Council (GMVEMSC)	**CHECKLIST VERSION / REVIEW DATE** DRAFT v0.1 · built 2026-10-30 · review date: _____

1 RECOGNITION / IMMEDIATE PRIORITIES

PRESENTATION (2008.2 ASSESSMENT)

☐	**EMR+**	Signs & symptoms: chest pain; shortness of breath; syncope; pallor, diaphoresis; radiation of pain; weakness; nausea; vomiting.	2008.2 Signs & Symptoms · p.59
	ALL	Listed differential: pericarditis; pulmonary embolism; asthma/COPD; pneumothorax; aortic dissection or aneurysm; GE reflux or hiatal hernia; chest trauma; esophageal spasm.	2008.2 Differential · p.59

INCLUSION / EXCLUSION / IMMEDIATE PRIORITIES

☐	**ALL**	Unstable cardiac patients are hypotensive, or have chest pain with poor skin color or diaphoresis.	2008.1.a · p.59
☐	**ALL**	Without consultation, this protocol applies only to patients greater than 25 years old with ACS symptoms. *Condition / note: 8004/8034 wording differs (see U-2008-1).*	2008.2 Consult · p.59
☐	**ALL**	PEDIATRIC: chest pain is rarely related to a cardiac event; complete assessment for other causes (e.g., muscle pain, respiratory difficulties, injury); apply supplemental oxygen and transport; THE REST OF THE CHEST PAIN ALGORITHM DOES NOT APPLY TO PEDS; contact MCP for further advice as needed.	2008.2 Ped. Considerations / Consult · p.59
☐	**ALL**	Pediatric = patients less than 16 years old.	1005.4 Ped. Considerations · p.21

2 ASSESSMENT

☐	**EMR+**	Apply 1005 General Patient Management; obtain chief complaint, OPQRST, SAMPLE history and other pertinent information; initial assessment / physical exam.	1005.4 Tx Algorithm, EMR block · p.22
☐	**EMR+**	Vital signs: blood pressure; pulse rate and quality; respirations (rate, quality, work of breathing); assess every 5 to 15 minutes per patient condition.	1005.4 Tx Algorithm, EMR block · p.22
☐	**EMR+**	Perform blood glucose check.	1005.4 Tx Algorithm, EMR block · p.22
☐	**EMR+**	Oxygen saturation threshold: less than 94% — oxygen via NC titrated to 94%; 94% or higher — no oxygen. Do not withhold oxygen from a patient with SOB or respiratory distress.	2008.2 Tx Algorithm, EMR block · p.59
☐	**EMT+** **{ } OPTION**	Prior to moving the patient, acquire a supine {12 Lead EKG} on all patients with ACS symptoms.	2008.2 Tx Algorithm, EMT block · p.59
☐	**EMT+**	Cardiac Alert criteria on a diagnostic 12-lead: greater than 1 mm ST elevation in 2 contiguous leads (AMI evidence). Excluded: Left Bundle Branch Block (QRS greater than 120 milliseconds); pacemaker rhythm.	2009.2.b, 2009.3.a-b · p.61
☐	**PM** **{ } OPTION**	The Paramedic is expected to read and interpret the {12-lead EKG}; do NOT rely solely on the computer interpretation or expect the physician to interpret the transmitted EKG for you.	2009.4 Consult · p.61

3 TREATMENT / CRITICAL ACTIONS

EMT LEVEL AND ABOVE — EACH ITEM BELOW NEEDS AN MCP ORDER FOR THE EMT

☐	**EMT+** **MCP ORDER (EMT)**	Aspirin (ASA) 324 mg chewed (four 81 mg tablets) to every patient greater than 25 y/o with symptoms of Acute Coronary Syndrome (ACS). Patient must chew the aspirin. *Condition / note: 8004: Medical Control for AEMT and Paramedic: No, unless patient is 25 y/o or younger with AMI symptoms.*	2008.2 Tx Algorithm, EMT block; Pearls · p.59 8004 Adult Dosing · p.231
☐	**EMT+** **MCP ORDER (EMT)**	Nitroglycerin 0.4 mg SL every 5 minutes for pain, to a total of three pills, with vital signs between doses. SBP must be greater than 100. Patient must be greater than 25 y/o. *Condition / note: 8034: Medical Control for AEMT and Paramedic: No. 8034 Precautions differ on age (see U-2008-1).*	2008.2 Tx Algorithm, EMT / AEMT blocks · p.59 8034 Adult Dosing · p.291

AEMT LEVEL AND ABOVE

☐	**AEMT+ MCP ORDER**	Patient 25 y/o or younger: MUST obtain MCP permission before administering Aspirin.	2008.2 Tx Algorithm, AEMT block · p.59 8004 Medical Control · p.231
☐	**AEMT+**	Prior to Nitroglycerin, establish vascular access for patients who have not previously had Nitroglycerin.	2008.2 Tx Algorithm, AEMT block · p.59
☐	**AEMT+**	Consider 1014 Pain Management (fentanyl) provided SBP is greater than 100 after the first dose of nitroglycerin. DO NOT WAIT UNTIL 3 NITROGLYCERIN TABLETS ARE GIVEN BEFORE CONSIDERING FENTANYL.	2008.2 Tx Algorithm, AEMT block · p.59 1014.2 AEMT block · p.41
☐	**AEMT+**	ADULT fentanyl (1014 / 8021): if SBP greater than 100, 50-100 mcg IV, may repeat 50-100 mcg IV after 5 minutes; if no IV, 50-100 mcg IN, SQ or IM, may repeat after 10 minutes. Patients greater than 69 y/o: reduce sedative and analgesic dosing to one half of the adult dose. *Condition / note: 1014 says 'IV'; 8021 says 'slow IV' (see U-2008-5). 8021 Medical Control: adults No.*	1014.2 AEMT block · p.41 8021 Adult Dosing · p.265
☐	**AEMT+**	Provide continuous cardiac monitoring, EtCO2 and pulse oximetry (if available) for all patients receiving fentanyl, ketamine, morphine or midazolam, if not already doing so.	1005.4 Tx Algorithm, AEMT block · p.22
☐	**ALL**	Ketamine is NOT to be administered to patients with suspected cardiac chest pain.	1014.1.d · p.41
☐	**AEMT+**	IV fluid, up to 500 ml, may be administered to a patient with SBP less than 100 without pulmonary edema.	2008.2 Tx Algorithm, AEMT block · p.59
☐	**AEMT+**	IV rate for cardiac issues with stable BP: use TKO rate.	1005.4 Tx Algorithm, AEMT block · p.22

PARAMEDIC

☐	**PM**	Treat cardiogenic shock with or without pulmonary edema as identified in 4016 Shock. *Condition / note: 4016 NOT reviewed / reproduced (see G-3).*	2008.2 Tx Algorithm, Paramedic block · p.59
☐	**PM { } OPTION**	The Paramedic should only transmit a {12/15 Lead EKG} that meets Cardiac Alert criteria, or that is questionable.	2008.2 Tx Algorithm, Paramedic block · p.59

CONTRAINDICATIONS AND CAUTIONS (2008 PEARLS, 8004, 8034)

☐	**EMT+**	ASPIRIN contraindications: hypersensitivity to salicylates; active ulcer disease; bleeding disorders; third trimester pregnancy. Aspirin is contraindicated in third trimester of pregnancy.	8004 Contraindications · p.231 2008.2 Clinical Pearls · p.59
☐	**EMT+**	NITROGLYCERIN contraindications: hypersensitivity; hypotension; sexual enhancement drugs (Viagra, Cialis, Levitra) in last 24 hours; taking Revatio; head injury. Do not administer NTG if the patient has taken Viagra, Cialis, Levitra, Revatio, or similar medications within the last 24 hours. *Condition / note: Revatio time window differs between sources (see U-2008-4).*	8034 Contraindications · p.291 2008.2 Clinical Pearls · p.59
	ALL	Pearl: no significant change in patient condition in the field should be expected from the administration of aspirin.	2008.2 Clinical Pearls · p.59

EMT DELEGATION — from the source level tables

MCP ORDER	Aspirin administration; Nitroglycerin administration; accessing the GMVEMSC Drug Bag (2008.2 Consult, PDF p.59; 8004 and 8034 Medical Control: EMTs Yes).
MAY	EMT: apply O2 as appropriate and per saturation thresholds; acquire supine {12 Lead EKG} prior to moving the patient; {transmit 12 Lead EKG} with two identifiers to MCP; consult MCP for destination (2008.2 EMR/EMT blocks).
MAY	EMT may assist the advanced provider, under direct supervision of the AEMT or Paramedic, with: IV access; IV fluid administration; saline locks; placement of 4 and/or {12/15 Lead EKG}; accessing the GMVEMSC Drug Bag to locate drugs and/or assemble pre-jects (1005.3, PDF p.21). The EMT may set up an IV administration kit in the presence of an AEMT or Paramedic (1005.4).
NOT LISTED AT EMT LEVEL	Vascular access before nitroglycerin, fentanyl (1014 AEMT block), IV fluid bolus for SBP less than 100, treatment of cardiogenic shock (4016), interpretation of the 12-lead EKG (2009.4 Paramedic expectation), norepinephrine.

4 REASSESSMENT / RESPONSE

☐	**EMT+**	Obtain vital signs between nitroglycerin doses.	2008.2 Tx Algorithm, EMT block · p.59
☐	**EMT+ { } OPTION**	Consider and transmit repeat {12 Lead EKGs} during transport (Cardiac Alert pathway: recommendation to repeat every 5 minutes or with any change in condition/presentation).	2008.2 Tx Algorithm, EMT block · p.59 2009.4 EMT block · p.61
☐	**PM**	Cardiac Alert pathway, Paramedic: reassess lungs frequently; monitor blood pressure and administer nitroglycerin or fentanyl cautiously; if significant bradycardia develops, utilize 2010 Bradycardia. *Condition / note: 2010 NOT reviewed (see G-3). 2009 Paramedic block also lists norepinephrine — NOT reproduced (8035 not reviewed).*	2009.4 Paramedic block · p.61
☐	**EMR+**	Vital signs every 5 to 15 minutes per patient condition (1005.4).	1005.4 Tx Algorithm, EMR block · p.22

Response-to-treatment criteria and a defined worsening / non-response pathway for chest pain (other than cardiogenic shock -> 4016): NOT SPECIFIED IN SOURCE — VERIFY.

5 TRANSPORT / NOTIFICATION / HANDOFF

TRANSPORT / DESTINATION

☐	**EMR+**	Arrange for rapid ALS transport.	2008.2 Tx Algorithm, EMR block · p.59
☐	**EMT+**	Consult MCP for appropriate destination.	2008.2 Tx Algorithm, EMT block · p.59
☐	**PM**	If evidence of STEMI: transport to an interventional cardiac catheterization lab.	2008.2 Tx Algorithm, Paramedic block · p.59 2009.4 EMT block · p.61
☐	**ALL**	Exceptions to transport to an interventional facility: medically necessary to go to the closest hospital for stabilization; unsafe to transport directly due to adverse weather/ground conditions or excessive transport time; transport would cause a critical shortage of local EMS resources; patient requests a different facility despite EMS education.	2009.4 Clinical Pearls · p.61
☐	**ALL**	Consider air medical transport if the interventional facility is over 30 minutes away.	2009.4 Clinical Pearls · p.61

NOTIFICATION / CONSULTATION

☐	**EMT+** **{ } OPTION**	{Transmit 12 Lead EKG} with two identifiers to MCP. The MCP shall be contacted after at least the initial 12-lead transmission is completed.	2008.2 Tx Algorithm, EMT block · p.59
☐	**AEMT+** **{ } OPTION**	AEMT must also transmit the {12 Lead EKGs}. EMT and AEMT should contact the MCP after 12-lead transmissions for further orders.	2008.2 Tx Algorithm, AEMT block · p.59 2009.4 Consult · p.61
☐	**ALL**	Cardiac Alert: providers make early notification to the receiving facility and speak directly with the physician; the physician may activate a Cardiac Alert based on provider impression and 12-lead interpretation. Request to speak directly to the MCP after a brief summary; state 'We recommend a ____ Alert'; the MCP may withhold the Alert.	2009.1.b-c · p.61 1002.3.c.ii · p.15

HANDOFF

☐	**ALL**	If possible, bring medications or a list of the medications to the hospital; include dose and frequency of administration.	1005.4 Clinical Pearls · p.22

Required verbal handoff / transfer-of-care elements for chest pain: NOT SPECIFIED IN SOURCE — VERIFY.

6 DOCUMENTATION / CLOSEOUT

☐	**ALL**	Document the patient's reported pain during initial patient contact, during treatment, and after any intervention.	1014.2 Clinical Pearls · p.41

Other required documentation elements, times, refusals, signatures: NOT SPECIFIED IN SOURCE — VERIFY.

7 SOURCE TRACEABILITY & REVIEW

Source citations: every checklist line shows its protocol section/paragraph and PDF page at the right. The full source-verification table for this protocol is in the SOURCE-VERIFICATION TABLES appendix. Full issue text is under REQUIRES HUMAN VERIFICATION.

Unresolved gaps / ambiguous wording / conflicts (do not infer or silently resolve)

ID	Source location	Gap (short form)
U-2008-1	2008.2 vs 8004 / 8034 (p.231)	Age: 2008 says 'greater than 25 y/o' (aspirin, nitroglycerin, protocol scope)
U-2008-2	2008.2 vs 8004 (p.231)	2008 gives aspirin for 'ACS symptoms'; 8004 Indications say 'Given as soon as possible to the patient with AMI'
U-2008-3	2008.2 vs 8034 (p.291)	Nitroglycerin: 2008 says 'SBP must be greater than 100'; 8034 lists 'Hypotension' with no number
U-2008-4	2008.2 Pearls vs 8034 (p.291)	2008: do not give NTG if Viagra, Cialis, Levitra, Revatio 'or similar medications' within the last 24 hours
U-2008-5	1014.2 vs 8021 (p.265)	Fentanyl route: 1014 says 'IV'; 8021 says 'slow IV'
U-2008-6	2008.2 Paramedic block (p.59)	Unstable cardiac patient is defined (2008.1.a) but only cardiogenic shock is directed to 4016
U-2008-7	2008.2 EMT block (p.59)	'{Transmit 12 Lead EKG} with two identifiers' — the two identifiers are not specified
U-2008-8	2009.4 Pearls (p.61)	2009 says interventional facilities are listed in '7013 Hospital Capabilities Chart', but the Table of Contents...
U-2008-9	2008.2 vs 2009 (p.59)	2008 was last modified Feb. 13, 2023 and 2009 Jan. 9, 2023; Appendix A shows the Paramedic 12-lead wording changed...
U-2008-10	2008.2 EMR block (p.59)	Oxygen: 'oxygen via NC and titrated to 94%'; no flow rate, device alternative or maximum is given.

Verification: [ ] Every clinical item traced to source [ ] Provider-level applicability checked [ ] Conflicts flagged

[ ] Reviewed by authorized clinical/agency reviewer: _____ Date: _____

REQUIRES HUMAN VERIFICATION

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

Global issues (all five protocols)

ID	Topic	Issue	Clarification needed
G-1	Edition / effective date	Cover (PDF p.1): 'Last Update 07/01/2026'. Appendix A (PDF p.321): '2026-2028 Protocol Changes', Effective Nov. 1, 2026. Each protocol prints 'Effective: June 1, 2021' and its own 'Last modified'. 1001.1.h: after release of a new version, all previous versions are invalid. Today is Oct. 30, 2026.	Confirm with agency clinical leadership which edition is in force today and on Nov. 1, 2026 before any field use.
G-2	Level mapping method	Provider levels come from the colored side bars of each Clinical Management Table (grey EMR, orange EMT, green AEMT, blue Paramedic). Text extraction cannot carry that information, so levels were read from rendered page images and cross-checked against Appendix A and the monographs.	Reviewer should spot-check level tags against the printed color bars.
G-3	Cross-referenced protocols not reproduced	Referenced but NOT built or reviewed in this batch: 1005 (only quoted lines used), 1008/1009 (airway), 1012 (IO), 2010 (bradycardia), 2011 (tachycardia), 4008, 4011, 4016 (shock), 8035 (norepinephrine), 7012/7013, 3019/3021.	Tell me which to add in later batches.
G-4	Agency options and Medical Director approval	Items in { } are agency/Medical Director options (1001.5.d.v) and 1001.4.d says nothing may be used without Medical Director pre-approval. This checklist cannot know which options your department has adopted. 1001.4.e-f: protocol is used as clinically indicated; not every standing order applies to every patient.	Agency Medical Director to mark adopted options.
G-5	Monograph level columns	Drug monographs (8000 series) print level tabs as column headers; the text extraction does not show which column each row sits in. Medication level statements in the checklist rely on the protocol tables, with monographs used for doses, contraindications and Medical Control lines.	Reviewer to confirm any level-specific monograph statement.
G-6	Status	This checklist has NOT been reviewed or approved by agency clinical leadership and does not authorize care. Reviewer sign-off lines in Section 7 are intentionally blank.	Complete the review sign-off.

1003 Non-Initiation of Care

ID	Source location	Issue	Clarification needed
U-1003-1	1003.3.a / 1003.2.a (p.17)	Exclusionary conditions 'will not meet non-initiation criteria' but the text does not say whether they override every 1003.2 criterion (e.g., a valid DNR, frozen body vs. 'signs of a hypothermic patient', or EMS-witnessed arrest with a valid DNR).	Medical Director: does any exclusion override 1003.2.a.iii (valid DNR) or .iv (frozen body)?
U-1003-2	1003.2.a.vii.1 (p.17)	'Prolonged arrest (greater than 10 minutes)' has no stated starting point (collapse, 911 call, EMS arrival?).	Define the start of the 10-minute interval.
U-1003-3	1003.1.c / 1002.2.b (p.17)	1003 allows resuscitation to cease without stating MCP contact; 2001 requires MCP approval for field termination. Relationship between 'cease' (1003.1.c) and 'field termination' (2001.2) is not stated.	Confirm whether MCP contact is required when stopping under 1003.1.c.
U-1003-4	1003.2.a.vi (p.17)	'Triage demands' is not defined in 1003 (SALT triage 3019 and Crisis Standard of Care 3021 exist; not reviewed for this checklist).	Confirm cross-reference and whether it should be cited.
U-1003-5	1003 (whole) (p.17)	1003 contains no documentation, assessment, notification or scene-disposition instructions. Last modified Feb. 11, 2024.	Confirm no companion policy (documentation, body disposition, law-enforcement notification) must be added.

1004 Do Not Resuscitate

ID	Source location	Issue	Clarification needed
U-1004-1	1004.1 / 1003.2.a.iii (p.19)	1003 uses 'DNR order present and valid' but 1004 does not describe what makes an order present or valid.	Agency: what does the crew physically verify (form, signature, bracelet/ID)?
U-1004-2	1004.2.a vs .b (p.19)	DNR-CC states it is initiated 'at the moment it is signed by the patient's physician'; no equivalent start statement is given for DNR-CCA.	Medical Director: confirm when a DNR-CCA takes effect.
U-1004-3	1004.3.a.iv (p.19)	BiPAP is listed as permitted for comfort; Appendix A (PDF p.321, 1007.1) shows BiPAP as a bracketed, agency-option item.	Confirm agency has approved BiPAP.
U-1004-4	1004.4.b (p.19)	'DPOA-HC supersedes the DNR if the DPOA-HC previously consented to the DNR' is ambiguous about what happens when the DPOA-HC revokes or disagrees.	Medical Director clarification; protocol also says to call MCP when confused (1004.4.g).
U-1004-5	1004.4.c (p.19)	Out-of-state and pediatric DNR orders: pediatric is defined as less than 16 years (1005.4); 1004 does not state how 16-17 year olds are handled.	Clarify age handling.
U-1004-6	1004 (whole) (p.19)	1004.4.g marks 'Call MCP for clarification' with a diamond (MCP order/contact). Treatment limits in 1004.3 apply 'once the DNR is initiated' / 'valid and effective'; no step-by-step sequence for when to stop CPR already underway is given (1003.1.c addresses only non-initiation criteria).	Confirm approach when a DNR is presented after resuscitation has begun.

2003 Cardiac Arrest: Asystole or PEA

ID	Source location	Issue	Clarification needed
U-2003-1	2003.2 vs 2002.2 / 2005.2 (p.49)	2003 says CPR 'for up to 2 minutes'; 2002 and 2005 say '1-2 minutes'. 2003 was last modified Oct. 10, 2021, while 2001 and 2002 were modified May 2026.	Confirm intended CPR interval and whether 2003 needs revision.
U-2003-2	2003.2 Paramedic block vs 2001.2 (p.49)	2003: 'Paramedic may consider Field Termination after administering Epinephrine' and 'no consult required unless applying Field Termination Guideline'. 2001: field termination 'requires MCP approval' and criteria (advanced airway, vascular access, etc.).	Confirm field termination is never done without MCP approval.
U-2003-3	2001.2 AEMT block (p.45)	'Upward trending or persistent EtCO2 >= 20 mmHg, refractory to VF or VT' is written for VF/VT, yet is listed in a protocol also used for asystole/PEA.	Does this EtCO2 criterion apply to asystole/PEA?
U-2003-4	2003.2 vs 8019 (p.261)	2003 says epinephrine 'IV or IO'; the 8019 dosing lines list 'IV' only. 1005.4 Pearls state any medication given intravenous can also be given IO.	Confirm IO is intended for epinephrine in arrest.

ID	Source location	Issue	Clarification needed
U-2003-5	2002.2 Pearls (p.47)	Hs & Ts are listed per certification level; whether the Paramedic applies all rows (cumulative) is not stated explicitly.	Confirm cumulative application.
U-2003-6	2001.2 / 2003.2 (p.45)	'Pediatric' is <16 years (1005.4); field termination requires 18 years or older (2001). Handling of ages 16-17 for dosing and other items is not stated.	Clarify for 16-17 year olds.
U-2003-7	2002.2 / 1008 / 1012 (p.47)	2002 conditions transport on the airway being secured and vascular access established; airway (1008/1009) and IO (1012) procedures are NOT reviewed or reproduced here.	Add airway/IO checklists in a later batch if desired.

2005 Cardiac Arrest: V-Fib or Pulseless V-Tach

ID	Source location	Issue	Clarification needed
U-2005-1	2005.2 vs 8003 (p.229)	2005 triggers amiodarone 300 mg after the THIRD defibrillation and 150 mg after the SIXTH; the 8003 monograph says repeat 150 mg 'no sooner than 10 minutes after first dose'. Not stated whether both conditions must be met.	Medical Director: is the 10-minute minimum in addition to the sixth-shock trigger?
U-2005-2	2005.2 Paramedic block (p.53)	'If Amiodarone is not available, use Lidocaine' — 'not available' is undefined. 8026 labels the V-Fib lidocaine use 'JITSO'; the term is not defined in the sections reviewed.	Define 'not available' and JITSO status.
U-2005-3	8003, 8029 Contraindications (p.229)	8003 lists contraindications (pulmonary congestion, cardiogenic shock, hypotension SBP less than 100). 8029 lists AMI, any heart block except 1st degree, abdominal pain, and caution if SBP less than 90. 2005 does not say whether these apply in pulseless arrest.	Medical Director: confirm applicability of monograph contraindications during cardiac arrest.
U-2005-4	2005.2 vs 8003 (p.229)	Post-ROSC amiodarone: 2005 says 'IV over 10 minutes using 60 drop/ml tubing'; 8003 adds 'wide open' and '18 g angiocath'. 8003 also requires continuous EKG monitoring.	Confirm which wording governs.
U-2005-5	2005.2 Consult vs 2001.2 (p.54)	2005 Consult says the AEMT/Paramedic may consult MCP to field terminate, but 2001 criteria permit field termination only in asystole or PEA with rates less than 40.	Confirm field termination is not available for VF/pulseless VT arrests.
U-2005-6	2005.3.a-c (p.54)	Vector Change and Double Sequential Defibrillation are bracketed {optional}; use requires explicit Medical Director consent. This checklist cannot tell whether your agency has approved them.	Agency Medical Director: approved or not?
U-2005-7	2005.2 (p.53)	Not specified: adult energy values (source says follow manufacturer); whether DSD/Vector Change shocks count toward the 'sixth defibrillation'; time to first epinephrine dose.	Medical Director clarification.
U-2005-8	2005.2 vs 8019 (p.261)	2005 says epinephrine 'IV or IO'; 8019 dosing lines list 'IV' only (1005.4 Pearls: any IV medication can be given IO).	Confirm IO is intended.
U-2005-9	2002.2 / 1008 / 1012 (p.47)	Airway (1008/1009) and IO (1012) procedures, and 2010/2011 rhythm protocols, are NOT reviewed or reproduced.	Add in later batches if desired.

2008 Suspected Cardiac Chest Pain

ID	Source location	Issue	Clarification needed
U-2008-1	2008.2 vs 8004 / 8034 (p.231)	Age: 2008 says 'greater than 25 y/o' (aspirin, nitroglycerin, protocol scope). 8004 Precautions say 'must be at least 25 years old'. 8034 Precautions say 'greater than 25 years old OR have been prescribed Nitroglycerin' — the prescribed-NTG pathway is not in 2008. A patient exactly 25 is treated differently.	Medical Director: exact age cutoff; is the prescribed-NTG pathway authorized?
U-2008-2	2008.2 vs 8004 (p.231)	2008 gives aspirin for 'ACS symptoms'; 8004 Indications say 'Given as soon as possible to the patient with AMI'. 8004 also cross-references 4011 Obstetrical Emergencies, which was not reviewed.	Confirm indication wording; review 4011 if pregnancy handling is needed.
U-2008-3	2008.2 vs 8034 (p.291)	Nitroglycerin: 2008 says 'SBP must be greater than 100'; 8034 lists 'Hypotension' with no number. 8034 also lists head injury and hypersensitivity; 2008 Pearls list only the erectile/pulmonary-hypertension drugs.	Confirm threshold and full contraindication list.
U-2008-4	2008.2 Pearls vs 8034 (p.291)	2008: do not give NTG if Viagra, Cialis, Levitra, Revatio 'or similar medications' within the last 24 hours. 8034: Revatio listed as 'Taking Revatio' with no time window; 'similar medications' is not defined.	Confirm window for Revatio and define 'similar'.
U-2008-5	1014.2 vs 8021 (p.265)	Fentanyl route: 1014 says 'IV'; 8021 says 'slow IV'. 1005.4 defines 'Slow IV = over 2 minutes, unless otherwise specified'.	Confirm administration time.
U-2008-6	2008.2 Paramedic block (p.59)	Unstable cardiac patient is defined (2008.1.a) but only cardiogenic shock is directed to 4016. 4016, 2010 (Bradycardia), 8035 (Norepinephrine), 1012 (IO) are NOT reviewed or reproduced.	Add in later batches or confirm they should be referenced only.
U-2008-7	2008.2 EMT block (p.59)	'{Transmit 12 Lead EKG} with two identifiers' — the two identifiers are not specified. 12-lead items are bracketed {agency option}.	Agency: which identifiers? Is 12-lead transmission adopted?
U-2008-8	2009.4 Pearls (p.61)	2009 says interventional facilities are listed in '7013 Hospital Capabilities Chart', but the Table of Contents lists 7012 as Hospital Capabilities Chart and 7013 as Hospital Contact Information.	Confirm correct cross-reference; the list of interventional facilities is NOT in this checklist.
U-2008-9	2008.2 vs 2009 (p.59)	2008 was last modified Feb. 13, 2023 and 2009 Jan. 9, 2023; Appendix A shows the Paramedic 12-lead wording changed to {12/15 Lead EKG} across the whole document (2008 Paramedic block reflects it; EMT/AEMT blocks and 2009 still print {12 Lead EKG}).	Confirm intended wording by level.
U-2008-10	2008.2 EMR block (p.59)	Oxygen: 'oxygen via NC and titrated to 94%'; no flow rate, device alternative or maximum is given.	Medical Director clarification if needed.

AUDIT SUMMARY

Automated source audit at build: 168 checklist items — every item has a section + PDF page citation and all key phrases were found in the cited pages (0 failures); 176 numeric tokens compared against cited pages (only protocol-number / section pointers are not printed on the cited page). 22 lines are marked NOT SPECIFIED IN SOURCE. Provider levels were read from the rendered color bars. Human review is still required: this checklist is NOT marked fully verified.

SOURCE-VERIFICATION TABLES

STATUS: DRAFT — NOT FOR CLINICAL USE UNTIL SOURCE-VERIFIED AND APPROVED THROUGH THE AGENCY'S PROCESS. This checklist does not itself authorize care.

One row per checklist line. 'Traced' = the cited section and PDF page were located and the item's key phrases and numbers were machine-matched to that page; levels were read from the rendered color bars. Human review is still pending. Level column: lowest level at which the source lists the item (cumulative).

1003 Non-Initiation of Care — PDF p.17

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
1003-1.01	Protocol may be applied by EMT, AEMT and Paramedic only. The...	1003.1.a (p.17)	EMT+	-	Traced; review pending
1003-1.02	All patients (adult, pediatric, geriatric) may meet...	1003.1.b (p.17)	ALL	-	Traced; review pending
1003-1.03	Deep, penetrating, cranial injuries.	1003.2.a.i (p.17)	EMT+	-	Traced; review pending
1003-1.04	Massive truncal wounds.	1003.2.a.ii (p.17)	EMT+	-	Traced; review pending
1003-1.05	DNR order present and valid (see 1004 Do Not Resuscitate).	1003.2.a.iii (p.17)	EMT+	-	Traced; review pending
1003-1.06	Frozen body.	1003.2.a.iv (p.17)	EMT+	-	Traced; review pending
1003-1.07	Rigor mortis, tissue decomposition, or severe dependent...	1003.2.a.v (p.17)	EMT+	-	Traced; review pending
1003-1.08	Triage demands.	1003.2.a.vi (p.17)	EMT+	-	Traced; review pending
1003-1.09	Arrest from BLUNT OR PENETRATING TRAUMA: CONSIDER not...	1003.2.a.vii (p.17)	EMT+	'Consider' language: discretionary, not mandatory.	Traced; review pending
1003-1.10	Traumatic arrest in a female patient with known pregnancy...	1003.3.a.i (p.17)	EMT+	-	Traced; review pending
1003-1.11	Possible medical etiology for the traumatic cardiac arrest.	1003.3.a.ii (p.17)	EMT+	-	Traced; review pending
1003-1.12	Arrest witnessed by EMS providers.	1003.3.a.iii (p.17)	EMT+	-	Traced; review pending
1003-1.13	Lightning strike.	1003.3.a.iv (p.17)	EMT+	-	Traced; review pending
1003-1.14	Signs or symptoms of a hypothermic patient.	1003.3.a.v (p.17)	EMT+	-	Traced; review pending
1003-1.15	Focused blunt trauma to the chest (commotio cordis).	1003.3.a.vi (p.17)	EMT+	-	Traced; review pending
1003-2.01	Decision threshold in trauma arrest: 'prolonged arrest' is...	1003.2.a.vii.1 (p.17)	EMT+	Starting point of the 10 minutes is not defined (see U-1003-2).	Traced; review pending
1003-2.02	Required primary / focused assessment, history, exam findings...	-		-	NOT SPECIFIED — VERIFY
1003-3.01	If a 1003.2 criterion is met and no 1003.3 exclusionary...	1003.2.a (p.17); 1003.3.a (p.17)	EMT+	Interaction of 1003.2 and 1003.3 is not spelled out (see U-1003-1).	Traced; review pending
1003-3.02	If care has already begun and it is readily apparent the...	1003.1.c (p.17)	EMT+	-	Traced; review pending
1003-3.03	Medication / procedure details: NOT SPECIFIED IN SOURCE —...	-		-	NOT SPECIFIED — VERIFY
1003-4.01	Reassessment, response criteria, worsening pathway: NOT...	-		-	NOT SPECIFIED — VERIFY
1003-5.01	Organ donation inquiry: direct the call to Life Connection of...	1003.4 (p.17)	ALL	-	Traced; review pending
1003-5.02	Reasons to contact Medical Control listed in 1002.2.b include...	1002.2.b (p.15)	ALL	1003 itself does not require MCP contact for non-initiation (see U-1003-3).	Traced; review pending
1003-5.03	Transport / destination, receiving-facility notification and...	-		-	NOT SPECIFIED — VERIFY
1003-6.01	Documentation / closeout elements for non-initiation: NOT...	-		-	NOT SPECIFIED — VERIFY

1004 Do Not Resuscitate — PDF p.19

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
1004-1.01	Consider and honor all valid Ohio Do Not Resuscitate orders...	1004.1.a (p.19)	ALL	-	Traced; review pending
1004-1.02	The two valid DNR orders are DNR: Comfort Care and DNR: Comfort...	1004.1.b (p.19)	ALL	-	Traced; review pending
1004-1.03	DNR Comfort Care Arrest (DNR-CCA): permits any GMVEMSC Protocol...	1004.2.a (p.19)	ALL	-	Traced; review pending
1004-1.04	DNR Comfort Care (DNR-CC): permits any medical treatment to...	1004.2.b (p.19)	ALL	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
1004-1.05	If more than one living will declaration or DNR exists, the...	1004.4.a (p.19)	ALL	-	Traced; review pending
1004-1.06	The authority of a DPOA-HC supersedes the DNR if the DPOA-HC...	1004.4.b (p.19)	ALL	-	Traced; review pending
1004-1.07	Out-of-state DNR orders and pediatric DNR orders are recognized...	1004.4.c (p.19)	ALL	-	Traced; review pending
1004-2.01	Conduct an initial assessment (permitted once DNR is initiated).	1004.3.a.i (p.19)	ALL	-	Traced; review pending
1004-2.02	How a DNR is recognized as 'present and valid' (document type...	-		-	NOT SPECIFIED — VERIFY
1004-2.03	Required history, exam findings, measurements and thresholds...	-		-	NOT SPECIFIED — VERIFY
1004-3.01	Perform basic medical care.	1004.3.a.ii (p.19)	ALL	-	Traced; review pending
1004-3.02	Clear airway of obstruction or suction.	1004.3.a.iii (p.19)	ALL	-	Traced; review pending
1004-3.03	If necessary, for comfort or to relieve distress: oxygen, CPAP...	1004.3.a.iv (p.19)	ALL	BiPAP appears as an agency-option item elsewhere (Appendix A, 1007.1) — see U-1004-3.	Traced; review pending
1004-3.04	If necessary, IV access for hydration or pain medication to...	1004.3.a.v (p.19)	ALL	-	Traced; review pending
1004-3.05	If possible, contact other appropriate health care providers.	1004.3.a.vi (p.19)	ALL	-	Traced; review pending
1004-3.06	Blood glucose checks and treatment of 4008 Diabetic Emergencies...	1004.4.d (p.19)	ALL	-	Traced; review pending
1004-3.07	Do NOT perform CPR.	1004.3.b.i (p.19)	ALL	-	Traced; review pending
1004-3.08	Do NOT administer resuscitation medications with the intent of...	1004.3.b.ii (p.19)	ALL	-	Traced; review pending
1004-3.09	Do NOT insert an airway adjunct.	1004.3.b.iii (p.19)	ALL	-	Traced; review pending
1004-3.10	Do NOT defibrillate, cardiovert or initiate pacing.	1004.3.b.iv (p.19)	ALL	-	Traced; review pending
1004-3.11	Do NOT initiate continuous cardiac monitoring.	1004.3.b.v (p.19)	ALL	-	Traced; review pending
1004-3.12	1005 requires continuous cardiac monitoring when administering...	1004.4.e (p.19)	ALL	-	Traced; review pending
1004-4.01	Reassessment timing, response criteria and worsening pathway...	-		-	NOT SPECIFIED — VERIFY
1004-5.01	Questions about the documents: try to keep the patient's intent...	1004.4.f (p.19)	ALL	-	Traced; review pending
1004-5.02	Any confusion on scene: call MCP for clarification.	1004.4.g (p.19)	ALL	-	Traced; review pending
1004-5.03	Reasons to contact Medical Control include 'field termination...	1002.2.b (p.15)	ALL	-	Traced; review pending
1004-5.04	Transport / destination and handoff content for DNR patients...	-		-	NOT SPECIFIED — VERIFY
1004-6.01	Documentation / closeout (copy of order, times, who was...	-		-	NOT SPECIFIED — VERIFY

2003 Cardiac Arrest: Asystole or PEA — PDF p.49 (+ 2001 p.45, 2002 p.47)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2003-1.01	Unresponsive; pulseless and apneic.	2003.2 Signs & Symptoms (p.49)	EMR+	-	Traced; review pending
2003-1.02	EITHER no electrical activity on the cardiac monitor OR...	2003.2 Signs & Symptoms (p.49)	EMR+	-	Traced; review pending
2003-1.03	Listed differential: ventricular fibrillation; pulseless...	2003.2 Differential (p.49)	ALL	-	Traced; review pending
2003-1.04	In ALL cardiac arrest patients apply 2002 Cardiac Arrest: Basic...	2003.1.a-b (p.49)	EMR+	-	Traced; review pending
2003-1.05	Rhythms may change: be flexible to move between the different...	2003.1.c (p.49)	EMR+	-	Traced; review pending
2003-1.06	Resuscitation applies when the patient does NOT meet the...	2001.2 Assessment (p.45)	ALL	-	Traced; review pending
2003-1.07	Pediatric: dosing should never exceed adult doses. Pediatric =...	2003.2 Ped. Considerations (p.49); 1005.4 Ped. Considerations (p.21)	ALL	-	Traced; review pending
2003-2.01	Apply the AED and check for a shockable rhythm.	2003.2 Tx Algorithm, EMR block (p.49)	EMR+	-	Traced; review pending
2003-2.02	Apply 1005 General Patient Management to all patients (not...	1005.1.a (p.21)	ALL	1005 content is NOT reproduced in this checklist (see G-3).	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2003-2.03	Required history, exam findings, measurements and numeric...	-		-	NOT SPECIFIED — VERIFY
2003-3.01	If witnessed or unwitnessed arrest: initiate quality CPR for up...	2003.2 Tx Algorithm, EMR block (p.49)	EMR+	2002 and 2005 state '1-2 minutes' (see U-2003-1).	Traced; review pending
2003-3.02	Follow the 2002 Cardiac Arrest - BLS protocol.	2003.2 Tx Algorithm, EMR block (p.49)	EMR+	-	Traced; review pending
2003-3.03	If no defibrillation is indicated: continuous CPR.	2003.2 Tx Algorithm, EMR block (p.49)	EMR+	-	Traced; review pending
2003-3.04	CPR parameters (adult): compressions 100 to 120 per minute...	2002.1 table (p.47)	EMR+	Pediatric, infant and newborn values: see 2002.1 table (not reproduced).	Traced; review pending
2003-3.05	Obtain and transmit {12 Lead EKG} if the patient has ROSC.	2003.2 Tx Algorithm, EMT block (p.49)	EMT+	-	Traced; review pending
2003-3.06	Consider possible causes.	2003.2 Tx Algorithm, AEMT block (p.49)	AEMT +	-	Traced; review pending
2003-3.07	Consider ACLS treatable causes (Hs & Ts) to your level of...	2002.2 Clinical Pearls (p.47)	EMR+	Cumulative application for the Paramedic is implied, not stated (see U-2003-5).	Traced; review pending
2003-3.08	ADULT: Epinephrine (1:10,000) 1 mg IV or IO, repeat every 3-5...	2003.2 Tx Algorithm, Paramedic block (p.49); 8019 Adult Dosing (p.261)	PM	8019 dosing line lists 'IV' only (see U-2003-4).	Traced; review pending
2003-3.09	PEDIATRIC: Epinephrine (1:10,000) 0.01 mg/kg IV or IO, repeat...	2003.2 Tx Algorithm, Paramedic block (p.49); 8019 Pediatric Dosing (p.261)	PM	-	Traced; review pending
2003-3.10	8019 Epinephrine 1:10,000: indicated 'For the Paramedic only'...	8019 Indications / Contraindications / Medical Control (p.261)	PM	-	Traced; review pending
2003-3.11	The Paramedic may consider Field Termination after...	2003.2 Tx Algorithm, Paramedic block (p.49)	PM	Field termination requires MCP approval and criteria in 2001 (see Section 5).	Traced; review pending
2003-3.12	Paramedics are expected to provide ALS resuscitative care at...	2001.1.b-c (p.45)	PM	-	Traced; review pending
2003-4.01	Change the person compressing the chest every 2 minutes.	2002.2 Clinical Pearls (p.47)	EMR+	-	Traced; review pending
2003-4.02	If ROSC: follow 2001 Resuscitation Guidelines.	2003.1.d (p.49)	EMR+	-	Traced; review pending
2003-4.03	Prolonged resuscitation efforts are required if: PEA with a...	2001.2 Tx Algorithm, AEMT block (p.45)	AEMT +	'Refractory to VF or VT' wording (see U-2003-3).	Traced; review pending
2003-4.04	Required reassessment timing (pulse/rhythm-check interval)...	-		-	NOT SPECIFIED — VERIFY
2003-4.05	Time to first epinephrine dose and criteria for stopping...	-		-	NOT SPECIFIED — VERIFY
2003-5.01	Cardiac arrests should NOT be transported unless: when...	2002.2 Tx Algorithm, Paramedic block (p.47)	PM	-	Traced; review pending
2003-5.02	ROSC: transport to an interventional facility if transport time...	2001.2 EMT block (p.45); 2002.2 EMT block (p.47)	EMT+	-	Traced; review pending
2003-5.03	A documented STEMI AND a witnessed cardiac arrest: transport to...	2001.2 Paramedic block (p.45)	PM	-	Traced; review pending
2003-5.04	Arrest due to profound hypothermia: rapidly transport to a...	2001.2 AEMT block (p.45)	AEMT +	-	Traced; review pending
2003-5.05	Consider aeromedical transport for transports greater than 30...	2001.2 Clinical Pearls (p.45)	ALL	-	Traced; review pending
2003-5.06	FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS.	2001.2 Ped. Considerations (p.45)	ALL	-	Traced; review pending
2003-5.07	Field termination requires MCP approval and may only be...	2001.2 AEMT block (p.45)	AEMT +	-	Traced; review pending
2003-5.08	When the AEMT or Paramedic contacts MCP for consent for field...	2001.2 Consult (p.45)	AEMT +	-	Traced; review pending
2003-5.09	No consult required unless applying the Field Termination...	2003.2 Consult (p.49)	AEMT +	-	Traced; review pending
2003-5.10	Contact ED to request a Cardiac Alert if applicable. When...	2003.2 Consult (p.49); 1002.3.c.ii (p.15)	ALL	-	Traced; review pending
2003-5.11	Contact the receiving hospital prior to arrival with a cardiac...	2003.2 Clinical Pearls (p.49); 1002.1.a.i (p.15)	ALL	-	Traced; review pending
2003-5.12	Required handoff / transfer-of-care content: NOT SPECIFIED IN...	-		-	NOT SPECIFIED — VERIFY
2003-6.01	Required documentation elements, times, refusals, signatures...	-		-	NOT SPECIFIED — VERIFY

2005 Cardiac Arrest: V-Fib or Pulseless V-Tach — PDF pp.53-54 (+ 2001 p.45, 2002 p.47; monographs 8003, 8019, 8026, 8029)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2005-1.01	Unresponsive; pulseless and apneic; ventricular fibrillation or...	2005.2 Signs & Symptoms (p.53)	EMR+	-	Traced; review pending
2005-1.02	Listed differential: asystole; artifact/device failure; signs...	2005.2 Differential (p.53)	ALL	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2005-1.03	In ALL cardiac arrest patients apply 2002 Cardiac Arrest: Basic...	2005.1.a-c (p.53)	EMR+	-	Traced; review pending
2005-1.04	If ROSC: follow 2001 Resuscitation Guidelines.	2005.1.d (p.53)	EMR+	-	Traced; review pending
2005-1.05	Resuscitation applies when the patient does NOT meet the...	2001.2 Assessment (p.45)	ALL	-	Traced; review pending
2005-1.06	Pediatric: dosing should never exceed adult doses. Pediatric =...	2005.2 Ped. Considerations (p.53); 1005.4 Ped. Considerations (p.21)	ALL	-	Traced; review pending
2005-1.07	REFRACTORY V-Fib/PVT = NOT CONVERTED by three standard...	2005.3.d.ii (p.54)	AEMT +	-	Traced; review pending
2005-1.08	RECURRENT V-Fib/PVT = V-Fib/PVT that reoccurs episodically...	2005.3.d.vi (p.54); 2005.3.f (p.54)	AEMT +	-	Traced; review pending
2005-2.01	Apply the AED / monitor and defibrillate as indicated by the...	2005.2 Tx Algorithm, EMR block (p.53)	EMR+	-	Traced; review pending
2005-2.02	AEMT level and above: defibrillate as required based on EKG...	2005.2 Tx Algorithm, AEMT block (p.53)	AEMT +	-	Traced; review pending
2005-2.03	Post-ROSC amiodarone decision point: do not infuse unless SBP...	2005.2 Tx Algorithm, Paramedic block (p.53)	PM	-	Traced; review pending
2005-2.04	Apply 1005 General Patient Management to all patients (not...	1005.1.a (p.21)	ALL	1005 content is NOT reproduced (see G-3).	Traced; review pending
2005-2.05	Required history, exam findings and measurements beyond rhythm...	-		-	NOT SPECIFIED — VERIFY
2005-3.01	If witnessed or unwitnessed arrest: initiate quality CPR for...	2005.2 Tx Algorithm, EMR block (p.53)	EMR+	-	Traced; review pending
2005-3.02	Follow the Basic Life Support protocol (2002).	2005.2 Tx Algorithm, EMR block (p.53)	EMR+	-	Traced; review pending
2005-3.03	Energy settings (adult): for initial and subsequent...	2005.2 Clinical Pearls (p.54)	EMR+	-	Traced; review pending
2005-3.04	PEDIATRIC energy: start at 2 J/kg (or biphasic equivalent) and...	2005.2 Clinical Pearls (p.54)	EMR+	-	Traced; review pending
2005-3.05	Obtain and transmit {12 Lead EKG} if the patient has ROSC.	2005.2 Tx Algorithm, EMT block (p.53)	EMT+	-	Traced; review pending
2005-3.06	Consider possible causes (Hs & Ts per 2002 Pearls; see 2003...	2005.2 Tx Algorithm, AEMT block (p.53); 2002.2 Clinical Pearls (p.47)	AEMT +	-	Traced; review pending
2005-3.07	ADULT: for refractory V-Fib/PVT after three shocks, consider...	2005.2 Tx Algorithm, AEMT block (p.53); 2005.3.d (p.54)	AEMT +	Optional skill; NOT to be applied without explicit Medical Director consent; adults only (2005.3.a-c).	Traced; review pending
2005-3.08	Alternate between CPR / defibrillation / medication...	2005.2 Tx Algorithm, Paramedic block (p.53)	PM	-	Traced; review pending
2005-3.09	ADULT: Epinephrine 1 mg 1:10,000 IV or IO, repeat every 3-5...	2005.2 Tx Algorithm, Paramedic block (p.53); 8019 Adult Dosing (p.261)	PM	8019 dosing line lists 'IV' only (see U-2005-8).	Traced; review pending
2005-3.10	PEDIATRIC: Epinephrine (1:10,000) 0.01 mg/kg IV or IO, repeat...	2005.2 Tx Algorithm, Paramedic block (p.53); 8019 Pediatric Dosing (p.261)	PM	-	Traced; review pending
2005-3.11	AFTER THIRD DEFIBRILLATION — ADULT: Amiodarone 300 mg IV or IO.	2005.2 Tx Algorithm, Paramedic block (p.53); 8003 Adult Dosing (p.229)	PM	-	Traced; review pending
2005-3.12	AFTER THIRD DEFIBRILLATION — PEDIATRIC: Amiodarone 5 mg/kg IV...	2005.2 Tx Algorithm, Paramedic block (p.53); 8003 Pediatric Dosing (p.229)	PM	-	Traced; review pending
2005-3.13	If Amiodarone is NOT available after third defibrillation —...	2005.2 Tx Algorithm, Paramedic block (p.53); 8026 Adult/Pediatric Dosing (p.275)	PM	'Not available' is not defined (see U-2005-2).	Traced; review pending
2005-3.14	ADULT, evidence of pulseless polymorphic VT (torsades de...	2005.2 Tx Algorithm, Paramedic block (p.53); 8029 Adult Dosing (p.281)	PM	-	Traced; review pending
2005-3.15	ADULT: after three traditional defibrillations and at least one...	2005.2 Tx Algorithm, Paramedic block (p.53); 2005.3.e (p.54)	PM	Optional skill; NOT to be applied without explicit Medical Director consent; adults only (2005.3.a-c).	Traced; review pending
2005-3.16	AFTER SIXTH DEFIBRILLATION — ADULT: Amiodarone 150 mg IV or IO...	2005.2 Tx Algorithm, Paramedic block (p.53); 8003 Dosing (p.229)	PM	8003: repeat 'no sooner than 10 minutes after first dose' (see U-2005-1).	Traced; review pending
2005-3.17	If Amiodarone is NOT available after sixth defibrillation —...	2005.2 Tx Algorithm, Paramedic block (p.53); 8026 Dosing (p.275)	PM	-	Traced; review pending
2005-3.18	Vector Change and Double Sequential Defibrillation are optional...	2005.3.a-c (p.54)	AEMT +	-	Traced; review pending
2005-3.19	VECTOR CHANGE (AEMT and Paramedic): for refractory V-Fib/PVT...	2005.3.d.i-vi (p.54)	AEMT +	-	Traced; review pending
2005-3.20	DOUBLE SEQUENTIAL DEFIBRILLATION (Paramedic): for refractory...	2005.3.e.i-v (p.54)	PM	-	Traced; review pending
2005-3.21	DSD CAUTION: every agency considering this procedure needs to...	2005.3.e.vi-vii (p.54)	PM	-	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2005-3.22	8003 AMIODARONE: adult 300 mg IV/IO, may repeat 150 mg IV/IO no...	8003 (p.229)	PM	-	Traced; review pending
2005-3.23	8026 LIDOCAINE (cardiac arrest V-Fib/Pulseless V-Tach, in the...	8026 (p.275)	PM	-	Traced; review pending
2005-3.24	8029 MAGNESIUM SULFATE (polymorphic VT / torsades, adult, with...	8029 (p.281)	PM	-	Traced; review pending
2005-4.01	Resume chest compressions immediately following each...	2005.2 Clinical Pearls (p.54)	EMR+	-	Traced; review pending
2005-4.02	Minimize interruptions to compressions before and after each...	2002.2 Clinical Pearls (p.47)	EMR+	-	Traced; review pending
2005-4.03	If patient converts with ROSC from a ventricular arrhythmia and...	2005.2 Tx Algorithm, Paramedic block (p.53); 8003 Adult Dosing (p.229)	PM	8003 adds 'wide open' and '18 g angiocath'; 2005 does not (see U-2005-4).	Traced; review pending
2005-4.04	If ROSC: follow 2001 Resuscitation Guidelines; obtain and...	2005.1.d, 2005.2 EMT block (p.53)	EMT+	-	Traced; review pending
2005-4.05	Refractory V-Fib/PVT (three standard defibrillations without...	2005.3.d-e (p.54)	AEMT +	-	Traced; review pending
2005-4.06	Required reassessment timing other than the intervals above...	-		-	NOT SPECIFIED — VERIFY
2005-5.01	Cardiac arrests should NOT be transported unless: when...	2002.2 Tx Algorithm, Paramedic block (p.47)	PM	-	Traced; review pending
2005-5.02	ROSC: transport to an interventional facility if transport time...	2001.2 EMT and Paramedic blocks (p.45); 2002.2 EMT block (p.47)	EMT+	-	Traced; review pending
2005-5.03	Consider aeromedical transport for transports greater than 30...	2001.2 Clinical Pearls (p.45)	ALL	-	Traced; review pending
2005-5.04	FIELD TERMINATION DOES NOT APPLY TO PEDIATRIC PATIENTS. For...	2001.2 Ped. Considerations / AEMT block (p.45)	AEMT +	2005 Consult mentions field termination but 2001 criteria name asystole/PEA only (see U-2005-5).	Traced; review pending
2005-5.05	No consult required unless applying the Field Termination...	2005.2 Consult (p.54)	AEMT +	-	Traced; review pending
2005-5.06	Contact ED to request a Cardiac Alert if applicable. When...	2005.2 Consult (p.54); 1002.3.c.ii (p.15)	ALL	-	Traced; review pending
2005-5.07	Contact the receiving hospital prior to arrival with a cardiac...	2005.2 Clinical Pearls (p.54); 1002.1.a.i (p.15)	ALL	-	Traced; review pending
2005-5.08	Required handoff / transfer-of-care content: NOT SPECIFIED IN...	-		-	NOT SPECIFIED — VERIFY
2005-6.01	Required documentation elements, shock counts, times, refusals...	-		-	NOT SPECIFIED — VERIFY

2008 Suspected Cardiac Chest Pain — PDF p.59 (+ 2009 p.61, 1014 p.41, 1005 pp.21-22; monographs 8004, 8021, 8034)

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2008-1.01	Signs & symptoms: chest pain; shortness of breath; syncope...	2008.2 Signs & Symptoms (p.59)	EMR+	-	Traced; review pending
2008-1.02	Listed differential: pericarditis; pulmonary embolism...	2008.2 Differential (p.59)	ALL	-	Traced; review pending
2008-1.03	Unstable cardiac patients are hypotensive, or have chest pain...	2008.1.a (p.59)	ALL	-	Traced; review pending
2008-1.04	Without consultation, this protocol applies only to patients...	2008.2 Consult (p.59)	ALL	8004/8034 wording differs (see U-2008-1).	Traced; review pending
2008-1.05	PEDIATRIC: chest pain is rarely related to a cardiac event...	2008.2 Ped. Considerations / Consult (p.59)	ALL	-	Traced; review pending
2008-1.06	Pediatric = patients less than 16 years old.	1005.4 Ped. Considerations (p.21)	ALL	-	Traced; review pending
2008-2.01	Apply 1005 General Patient Management; obtain chief complaint...	1005.4 Tx Algorithm, EMR block (p.22)	EMR+	-	Traced; review pending
2008-2.02	Vital signs: blood pressure; pulse rate and quality...	1005.4 Tx Algorithm, EMR block (p.22)	EMR+	-	Traced; review pending
2008-2.03	Perform blood glucose check.	1005.4 Tx Algorithm, EMR block (p.22)	EMR+	-	Traced; review pending
2008-2.04	Oxygen saturation threshold: less than 94% — oxygen via NC...	2008.2 Tx Algorithm, EMR block (p.59)	EMR+	-	Traced; review pending
2008-2.05	Prior to moving the patient, acquire a supine {12 Lead EKG} on...	2008.2 Tx Algorithm, EMT block (p.59)	EMT+	-	Traced; review pending
2008-2.06	Cardiac Alert criteria on a diagnostic 12-lead: greater than 1...	2009.2.b, 2009.3.a-b (p.61)	EMT+	-	Traced; review pending
2008-2.07	The Paramedic is expected to read and interpret the {12-lead...	2009.4 Consult (p.61)	PM	-	Traced; review pending
2008-3.01	Aspirin (ASA) 324 mg chewed (four 81 mg tablets) to every...	2008.2 Tx Algorithm, EMT block; Pearls (p.59); 8004 Adult Dosing (p.231)	EMT+	8004: Medical Control for AEMT and Paramedic: No, unless patient is 25 y/o or younger with AMI symptoms.	Traced; review pending

#	Checklist item (shortened)	Source section · PDF page	Level	Conditions / exceptions	Status
2008-3.02	Nitroglycerin 0.4 mg SL every 5 minutes for pain, to a total of...	2008.2 Tx Algorithm, EMT / AEMT blocks (p.59); 8034 Adult Dosing (p.291)	EMT+	8034: Medical Control for AEMT and Paramedic: No. 8034 Precautions differ on age (see U-2008-1).	Traced; review pending
2008-3.03	Patient 25 y/o or younger: MUST obtain MCP permission before...	2008.2 Tx Algorithm, AEMT block (p.59); 8004 Medical Control (p.231)	AEMT +	-	Traced; review pending
2008-3.04	Prior to Nitroglycerin, establish vascular access for patients...	2008.2 Tx Algorithm, AEMT block (p.59)	AEMT +	-	Traced; review pending
2008-3.05	Consider 1014 Pain Management (fentanyl) provided SBP is...	2008.2 Tx Algorithm, AEMT block (p.59); 1014.2 AEMT block (p.41)	AEMT +	-	Traced; review pending
2008-3.06	ADULT fentanyl (1014 / 8021): if SBP greater than 100, 50-100...	1014.2 AEMT block (p.41); 8021 Adult Dosing (p.265)	AEMT +	1014 says 'IV'; 8021 says 'slow IV' (see U-2008-5). 8021 Medical Control: adults No.	Traced; review pending
2008-3.07	Provide continuous cardiac monitoring, EtCO2 and pulse oximetry...	1005.4 Tx Algorithm, AEMT block (p.22)	AEMT +	-	Traced; review pending
2008-3.08	Ketamine is NOT to be administered to patients with suspected...	1014.1.d (p.41)	ALL	-	Traced; review pending
2008-3.09	IV fluid, up to 500 ml, may be administered to a patient with...	2008.2 Tx Algorithm, AEMT block (p.59)	AEMT +	-	Traced; review pending
2008-3.10	IV rate for cardiac issues with stable BP: use TKO rate.	1005.4 Tx Algorithm, AEMT block (p.22)	AEMT +	-	Traced; review pending
2008-3.11	Treat cardiogenic shock with or without pulmonary edema as...	2008.2 Tx Algorithm, Paramedic block (p.59)	PM	4016 NOT reviewed / reproduced (see G-3).	Traced; review pending
2008-3.12	The Paramedic should only transmit a {12/15 Lead EKG} that...	2008.2 Tx Algorithm, Paramedic block (p.59)	PM	-	Traced; review pending
2008-3.13	ASPIRIN contraindications: hypersensitivity to salicylates...	8004 Contraindications (p.231); 2008.2 Clinical Pearls (p.59)	EMT+	-	Traced; review pending
2008-3.14	NITROGLYCERIN contraindications: hypersensitivity; hypotension...	8034 Contraindications (p.291); 2008.2 Clinical Pearls (p.59)	EMT+	Revatio time window differs between sources (see U-2008-4).	Traced; review pending
2008-3.15	Pearl: no significant change in patient condition in the field...	2008.2 Clinical Pearls (p.59)	ALL	-	Traced; review pending
2008-4.01	Obtain vital signs between nitroglycerin doses.	2008.2 Tx Algorithm, EMT block (p.59)	EMT+	-	Traced; review pending
2008-4.02	Consider and transmit repeat {12 Lead EKGs} during transport...	2008.2 Tx Algorithm, EMT block (p.59); 2009.4 EMT block (p.61)	EMT+	-	Traced; review pending
2008-4.03	Cardiac Alert pathway, Paramedic: reassess lungs frequently...	2009.4 Paramedic block (p.61)	PM	2010 NOT reviewed (see G-3). 2009 Paramedic block also lists norepinephrine — NOT reproduced (8035 not reviewed).	Traced; review pending
2008-4.04	Vital signs every 5 to 15 minutes per patient condition...	1005.4 Tx Algorithm, EMR block (p.22)	EMR+	-	Traced; review pending
2008-4.05	Response-to-treatment criteria and a defined worsening /...	-		-	NOT SPECIFIED — VERIFY
2008-5.01	Arrange for rapid ALS transport.	2008.2 Tx Algorithm, EMR block (p.59)	EMR+	-	Traced; review pending
2008-5.02	Consult MCP for appropriate destination.	2008.2 Tx Algorithm, EMT block (p.59)	EMT+	-	Traced; review pending
2008-5.03	If evidence of STEMI: transport to an interventional cardiac...	2008.2 Tx Algorithm, Paramedic block (p.59); 2009.4 EMT block (p.61)	PM	-	Traced; review pending
2008-5.04	Exceptions to transport to an interventional facility...	2009.4 Clinical Pearls (p.61)	ALL	-	Traced; review pending
2008-5.05	Consider air medical transport if the interventional facility...	2009.4 Clinical Pearls (p.61)	ALL	-	Traced; review pending
2008-5.06	{Transmit 12 Lead EKG} with two identifiers to MCP. The MCP...	2008.2 Tx Algorithm, EMT block (p.59)	EMT+	-	Traced; review pending
2008-5.07	AEMT must also transmit the {12 Lead EKGs}. EMT and AEMT should...	2008.2 Tx Algorithm, AEMT block (p.59); 2009.4 Consult (p.61)	AEMT +	-	Traced; review pending
2008-5.08	Cardiac Alert: providers make early notification to the...	2009.1.b-c (p.61); 1002.3.c.ii (p.15)	ALL	-	Traced; review pending
2008-5.09	If possible, bring medications or a list of the medications to...	1005.4 Clinical Pearls (p.22)	ALL	-	Traced; review pending
2008-5.10	Required verbal handoff / transfer-of-care elements for chest...	-		-	NOT SPECIFIED — VERIFY
2008-6.01	Document the patient's reported pain during initial patient...	1014.2 Clinical Pearls (p.41)	ALL	-	Traced; review pending
2008-6.02	Other required documentation elements, times, refusals...	-		-	NOT SPECIFIED — VERIFY